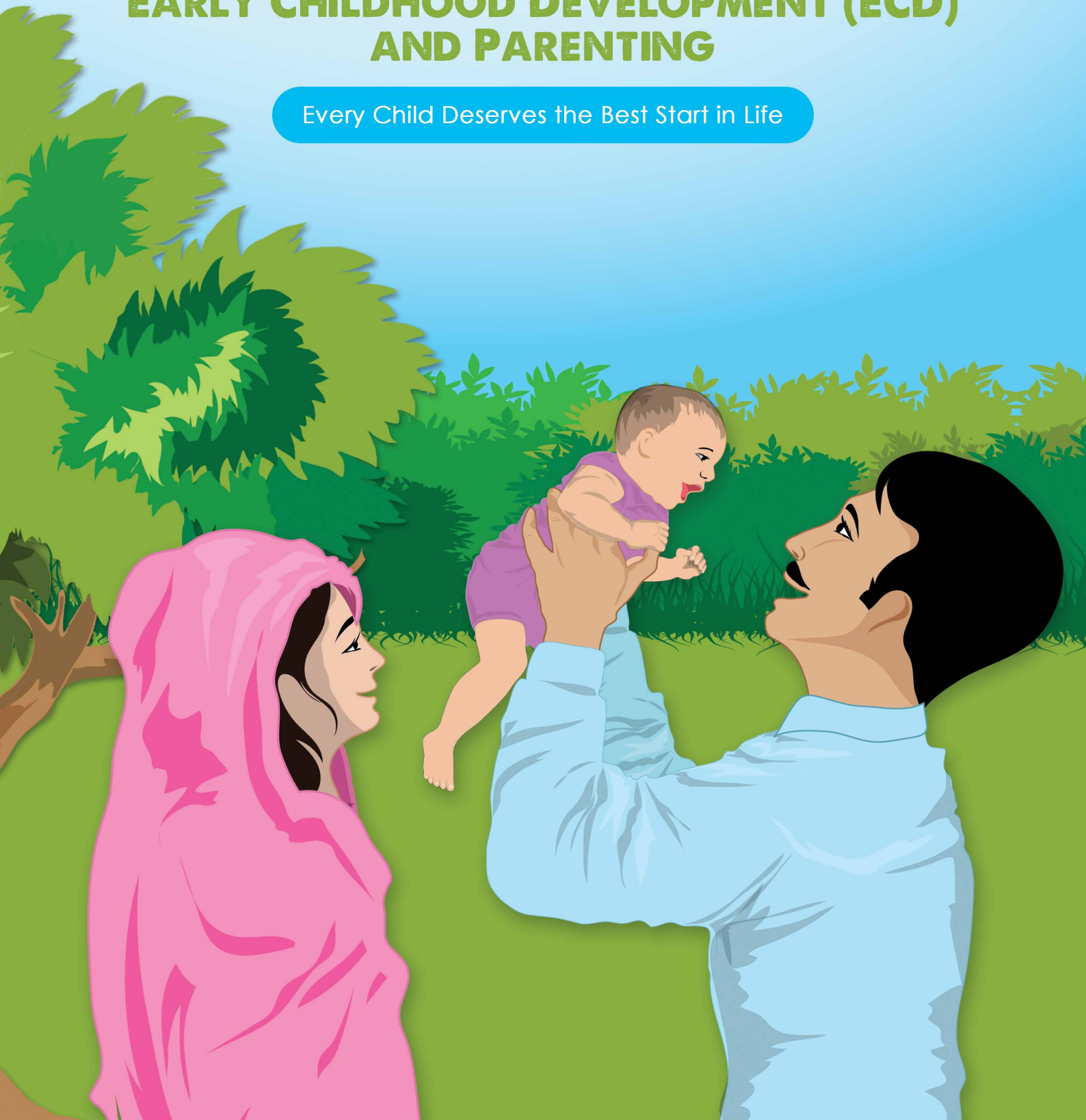




TRAINING MANUAL ON EARLY CHILDHOOD DEVELOPMENT (ECD) AND PARENTING

Every Child Deserves the Best Start in Life



Training Manual on Early Childhood Development (ECD) and Parenting

Every Child deserves the best start in life





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Foreword

It is the wish of every parent to bring up a healthy child to be a productive and responsible member of the society. If parents have the knowledge and enhanced skills in nurturing their children, and also recognize the needs and nourish them, they will be able to provide optimal care for holistic child development.

The Parenting Package has been developed under the leadership of the Nutrition Unit, SUN Secretariat, Ministry of Planning, Development, and Special Initiatives with support from UNICEF Pakistan, following consultations with multisectoral stakeholders at federal and provincial levels. The package is based on life cycle-based approach and covers the period from conception to 8 years, and focused on 22 Key Family Care Practices (KFCEPs), which are a key set of high impact and easy-to-change behaviors at the family and community levels for improving child health, nutrition and development. The topics cover health, nutrition, Water, Sanitation and Hygiene, child protection and education as well as nurturing care, Care for Child Development and Caring for the caregiver. Men's role and engagement in child development has been highlighted as critical and is promoted across the 22 practices.

The objective of parental education is to improve the capacity of parents, families/care givers, practitioners and communities on Early Childhood Development (ECD), nurturing and responsive caregiving. It will support parents, caregivers, family and community to provide positive parenting, nurturing care, integrated care to young children, which will ultimately contribute to holistic child survival, protection and development.

The Parenting Package consists of a Training manual, a Participant's handbook, Counselling cards on Key Family Care Practices and fliers on childhood development milestones. The package was pre-tested with trainers, frontline workers, health workers and communities for validity and reliability, and finalized.

It is expected that the Parenting Package will be used by multi-sectoral stakeholders from various Government departments, Civil Society Organizations as well as Community and Faith-based Organizations reaching families and communities. With the intention of educating parents/caregivers families and communities on early childhood development and positive parenting, the package will be rolled out across Pakistan through existing Government structures, and civil society organizations.



Humayun

Secretary, Planning and Development Department
Government of Khyber Pukhtunkhwa.



INTRODUCTION:

WELCOME to the Parenting Manual which is part of the Parenting Package to support parents, caregivers, family and community to provide integrated care to young children. This training module is going to inform parents and caregivers, teachers, social welfare workers, and all those paraprofessionals working with mothers and children on the needs of children and how to respond to them.

The introduction provides an overview of evidence-based concepts that informs the parenting manual. The purpose of this section is to define the elements of positive parenting and key family care practices for health and optimal child development.

SESSION OUTLINES OF THE MANUAL CONSIST OF:



Intended Audience for the Training Manual

This training manual is designed for the trainers of Lady Health Worker (LHW)/Other front-line workers and can be used by different levels of trainers (provincial, district, facility) with minor adaptations. There are two basic tenets of this training. One: the LHWs/other frontline worker gains sufficient knowledge of early childhood development, parenting and Key Family care Practices (KFCEPs) and two: she/he attains skills for the basics of interpersonal communication. The earlier empowers the LHWs/other frontline worker with information and the latter makes her skillful in engaging with mother and family to discuss problems and look for solutions.

Practical tips are provided for the trainer to organize training, conduct sessions and ensure hands-on skills to the LHWs/other frontline workers.

Duration of the training

The modules consists of several sessions of maximum 4 hours each although the time frame maybe adapted to fit the needs of the trainees in context.

Duration of the training will be 3 days for training of trainers including a one day of practices exercises.

Expected number of participants:

25-30 participants will be included in the training at each particular time.

No. of facilitators/trainers: (2/ or depends on No. of trainees)

How to use the Parenting Manual

In each session facilitator's note and participants notes provide session-by session instructional notes. It includes both the content of the course in the form of text and activities. Activities are found in boxes with the session and include instructions for participants to understand the activity either as individual, pair or in a group. Additional information that the facilitator will need to use as part of the preparations for the training session are also included as background information, facilitator's notes and handouts.

ICONS used in the manual and their meaning



Duration



Background information



Material needed



Key messages



Handouts

CONDUCTING THE TRAINING

The manual provides guidance to the trainers on organizing, conducting and supervising the trainings at different levels. The training will provide necessary information and practical guidelines to nurturing care and key family care practices for maximizing the physical, cognitive, socioemotional and cultural development.

After completing this training, the LHWs/other frontline workers such as Family welfare workers, Health and Nutrition Workers or Early Childhood Development (ECD) and Early Childhood Care and Education (ECCE) facilitator or others will use the counseling cards during their household visits.

Overall objective of the training:

The objective of this training is to improve the capacity of LHW/other frontline workers to increase their understanding of Early Childhood Development (ECD) to improve effective communication and counseling. After this training, LHW/other frontline workers will be able to provide education on parenting and responsive care to bring a positive change in people's behaviors towards holistic child survival, development and protection following a life course approach.

Training approach:

This training contains a new counseling approach, and appropriately timed messages on 22 key family care practices that LHW/other frontline worker will deliver in the family/community with the help of counseling cards.

Following are the key steps that trainers need to ensure:

1. Know the concepts
2. Follow the steps
3. Use slides and materials
4. Use energizers
5. Emphasize role plays
6. Handouts (where necessary)

Know the concepts

Early brain development has its foundation on two components:

- A. The trainers will need to have good grasp of key concepts of ECD and responsive care giving, which will be the core foundation, based on which the family care practices have been designed.
- B. Key Family Care Practices: This set of 22 KFCPs is the basic knowledge for LHW/other frontline workers. To empower the LHW/other frontline worker with this knowledge, the trainers themselves need to have a grasp of the importance of these practices and the ways these practices can be promoted among family and community.
- C. Knowledge only, however useful, is not enough for initiating and sustaining new behaviors. Front-line workers need to effectively communicate about these behaviors with their community. Knowing the context in which a certain behavior may not be happening and providing solutions for barriers if any, is an important step. Moreover, helping mother and family

in taking the first step towards a seemingly difficult new behavior, is also required. The simple technique of Ask, Brainstorm, Coach (ABC) of communication is a powerful way of talking to families and communities. The trainers should read, understand and practice these 3 steps during the training so that they can be confident in helping LHW/other frontline workers learn this effective way of persuasive communication.

Follow the steps

There are four main steps for each session of the training. These include:

- A. Introduce the topic, the session objectives, and present the content with the help of power point slide or flip chart.
- B. Conduct an interactive discussion. Ensure participation of all trainees, listen to their experiences and encourage discussion.
- C. Provide scenario and have the participants do a role-play where relevant. The scenario is provided in some sessions and give instructions for role-play.
- D. Feedback the role-play and conclude the session. Once role-play finishes, ask the group to provide their feedback. Add your own comments in the end and then conclude the session.

Use slides and materials

A good trainer makes the maximum use of training materials. Enumerate few culturally appropriate examples-names. The training material will consist of the manual, counselling cards, flyers and other relevant material. A set of power point slides is provided for you to use in your session. You can copy the points from slides on flipcharts and use them if power point presentation is not possible due to lack of equipment or electricity. You may also think of props (e.g., a doll or a home-made rattle) to add color and interest to your training sessions.

Use Energizers

Energizers are short activities that encourage playfulness and laughter. These usually involve physical movement and can either be used at the beginning of a session to illustrate a focus or encourage unity or later in the session as a break to 'energize' participants who may be tired, uncomfortable from sitting too long or simply tense from a lengthy discussion.

Role Plays

Role Play is a way of acting out particular scenarios. It is an effective way to prepare for unfamiliar or unexpected situation. By acting out scenarios, one can explore how other people are likely to respond to different situations. Role-plays also help in brainstorming and improving communication between two individuals or group of people. It builds up self-confidence and prepares for handling unexpected situations in real life. Through role-plays, LHW/other frontline workers will get to practice hands on skills throughout the training.

Handouts: Sessions have handouts for participants with key messages that is critical for this training and the trainer should make sure there are enough copies for participants.



Tips for the Facilitators

After each session, the facilitator should write down some notes about lessons learnt or other interesting discoveries made during the session which will help in the next training session.

Before the training, the trainer must:

1. Read the trainer manual beforehand.
2. Understand the structure and flow of each session.
3. Have both counseling cards and training manual with him/her at the time of training.
4. Ensure availability of the materials and supplies that will be used during training sessions e.g., flip charts, markers, writing pads, chart papers, panaflex or projector, etc.
5. Organize the seating arrangement for training session before time.
6. Prepare and organize slides and flip charts before every session and explain them in detail.
7. Allow every participant an equal chance to share his/her views and listen to them carefully.
8. Create a friendly environment and use three steps of ABC counseling during each session.
9. Provide positive feedback and try to avoid negative criticism.
10. Follow the agenda of training.

Training Sessions:

This training is organized in three components. A training program spanning over three days is proposed. Three modules are designed. Module 1 will be on introduction to the concept of ECD, parenting, nurturing and care, family relations and child care and development. Module 2 will focus on enhancing the counselling skills and module 3 will be on taking the participants through practical sessions on how to use key family care practices.

CONTENTS

Foreword.....	05
Introduction.....	06
Conducting the training	09
Tips for the facilitators.....	11
Contents	12
Module 1: Basic Concepts	13
Session 1: Early Childhood Development and its Importance	14
Session 2: Introduction to nurturing care and positive parenting	18
Session 3: The Family and harmonious relationship	22
Session 4: Building brain in Pakistan as foundation for lifelong development	26
Module 2: Counselling skills	29
Session 1: Counselling skills	30
Module 3: Key Family Care Practices	37
a) Spousal communication	40
b) Interval/ Spacing between Births	43
c) Antenatal care	47
d) Skilled birth attendance	51
e) Breastfeeding	54
f) Essential Newborn care	59
g) Danger signs in Newborn	62
h) Birth registration	65
i) Complementary feeding	68
j) Vaccination	73
k) Adolescent & Maternal Nutrition	78
l) Use of bed nets	81
m) Taking care of a sick child	84
n) Stimulation and Play	88
o) Men's engagement	94
p) Maternal mental health and wellbeing	97
q) Water sanitation and hygiene	100
r) Growth Monitoring	104
s) Child abuse and neglect	110
t) Child injuries	114
u) Positive discipline	118
v) School enrollment	121

MODULE 1



SESSION 1

EARLY CHILDHOOD DEVELOPMENT (ECD) AND ITS IMPORTANCE

Session 1

EARLY CHILDHOOD DEVELOPMENT (ECD) AND ITS IMPORTANCE



Background:

During the past decade, Pakistan has made significant progress in the area of mother and child health. Yet, the number of mothers and children dying is unacceptably high. It is estimated that one woman dies every hour, due to pregnancy and birth-related causes. The number of children dying is even higher. One child dies every minute in Pakistan. Many of those who survive face a plethora of problems. Diarrhea, pneumonia and malnutrition are common, and enough to pull back on likely achievements of those who manage to survive.

Owing to multiple factors including low birth weight, lack of proper feeding, and repeated illnesses, the boys and girls stop growing. From the outside, we see this in the form of child being less than normal in weight and short in height. Inside, the effects are not visible. It affects the child's brain development. The child faces difficulty in learning, gets low scores in school and ends up in low-income professions. In the adult life, this individual will earn less and provide insufficient food and opportunities to the family, transferring same problems to the next generation.

The hospitals, doctors and other medical staff have important roles to play. But equally important are the families, where the boys and girls, and their mothers, spend most of their time. Parents and other members of the family can take small steps to ensure that children are well cared for at homes and develop to their full potential both physically emotionally.

If families are educated on key family care practices, coached and motivated to practice them, it can help in ensuring optimum growth and development of children. The role of parents/caregivers, family and community is critical.



Rationale:

In order to understand the care of young children, participants should understand the concept of early childhood and the critical and sensitive period of child development; early childhood development, the integrated nature of child development and the different components of nurturing care for holistic development of children, key family care practices and why parents should give children nurturing and optimal care at this critical period of human development.



Learning Objectives

Upon completion of this session participants should be able to:

- Explain what is early childhood development.
- Understand the critical situation of young children in Pakistan with respect to indicators.
- Explain the importance of investing in comprehensive early childhood development care services.



Methodology:

Presentation, brainstormings, group discussions.



Material needed:

Flip chart, markers, power point slides, Handouts



Time duration: 90 mins

Activity 1: Concept of early childhood and ECD (30 minutes)

1. Divide the participants into 4-5 groups and assign each different question below as a guide.
 - a) Ask participants to discuss in groups on:
 - Concept of the child in Pakistan context.
 - What are parental expectations of a child (0-8 years of age) in Pakistan?
 - What should parents do to ensure that these expectations are met?
2. Ask participants to write answers on a flip chart.
3. Each group should nominate one group member to present their responses to the group questions.
4. Discuss the point and highlight early childhood as an integrated concept and summarize.

Activity 2: Situation of young children in Pakistan with respect to indicators (30 mins)

1. Give a projection of information on critical indicators: Population of Pakistan population of <5, No. of births, Under five morbidity and mortality rate, stunting and malnutrition rates, underweight and wasting, initiation of breastfeeding after birth, exclusive breastfeeding and LBW/preterm (use training cards from sections for a better understanding of the situation-Province specifics where training is taking place)
2. Give participants time for questions after the presentation.
3. Discuss child development in relation to the situation presented.
4. Ask the participants which are the indicators we are doing well on and which are the ones we are not doing well in.
5. Write the responses on a flip chart and discuss responses with the participants and summarize.
6. Answer any questions the participants may have.

Activity 3: Integrated early childhood development and key components (30 mins)

1. Ask participants what children need to grow well/optimally. Write the responses on a flip chart.
2. Draw a round circle with a holistic child development in the middle and connect different responses to the circle.

3. Discuss the responses and emphasise how integrated child care is important for holistic child development.
4. Summarize the main points of integrated ECD.

KEY MESSAGES

- Child development is the period from conception (pregnancy) to primary school entry age (0-8 years)
- Early Childhood development (ECD) consists of integrated services such as good health, balanced nutrition, safety and security of child, clean and good parent-child interaction with lots of play and communication.
- Ensure your child has these services to be healthy, perform well in school, like school with reduced repetition of class, complete school to job, and become a good citizen, and will be able to take care of his/her own family.
- From conception to three years of life are very important in the provision of nurturing care because more than 80% of brain develops during this time and is a critical period.



Note for the facilitator:

- ▶ Early childhood development includes physical, socio emotional, cognitive and motor development between 0-8 years of age.
- ▶ The early years are critical because child's brain develops rapidly and has a high capacity for change. This time lays foundation for the future. If a child is provided a favorable and nurturing environment, his/her chances to a better future increase.
- ▶ It is estimated that one woman dies every hour in Pakistan, due to pregnancy and birth-related causes. The number of children dying is even higher. One child dies every minute in our country. Those who survive usually face a plethora of problems. Diarrhea, pneumonia and malnutrition are common.
- ▶ If a child is provided with nurturing environment, his/her chances to survival and growth improve. It is the role of parents to provide their children with a nurturing environment that is sensitive to children's health and nutritional needs, provide protection from threats, offer opportunities for early learning, and allow interactions that are responsive, emotionally supportive and developmentally stimulating.
- ▶ The environment of a household plays a crucial role in a child's health and development. Safe and secure structures of the house, adequate amount and type of food, and availability of books and play materials - all have a key role in a child's life. Equally if not more important are the social and psychological factors.
- ▶ The love and affection of elders and the early learning through play with siblings are critical. Together, this nurturing care helps a child grow, learn and develop into a positive personality.



SESSION 2

INTRODUCTION TO NURTURING CARE AND POSITIVE PARENTING

Session 2:

INTRODUCTION TO NURTURING CARE AND POSITIVE PARENTING



Background:

Nurturing and care and responsive care giving helps children to develop to their full potential. Parents play a critical role in facilitating early experiences through the provision of nurturing care, defined as care which ensures health, nutrition, responsive caregiving, safety and security, social, emotional well-being, and early learning. The love and affection of elders and the early learning through play with siblings are critical. Together, this nurturing care helps a child grow, learn and develop into a positive personality. Parents should engage and interact with children and help children grow to their full potential.

Care for Child Development:

The Care for Child Development (CCD) intervention is designed to promote cognitive, motor, social and affective development for infants and young children through early stimulation and supports interactions between caregiver and child that build the caregiver's capacity to be sensitive and responsive. CCD is about play and communication with the child (Newborn, infant, child up to 2 years) in different contexts where the parent/caregiver and child are together. It can be during a structured play session, routine activities at home, when changing the child, feeding, teaching task etc. It involves skin-to-skin contact, hugging, face-to face with smiling laughing, facial mimics, message use of object in play, communication, showing love and praise to motivate the child. The CCD intervention should be integrated into existing programmes, including health, nutrition, education and child protection services.



Rationale:

Often times, the parents need help and guidance on how to provide nurturing care to their children. Different methods (e.g., using mass media) can be adopted for this guidance but reaching and helping them through community-based health workers is mostly recommended. In rural Pakistan, the Lady Health Worker (LHW) of the National Programme for Family Planning and Primary Health Care, popularly called Lady Health Worker Program (LHWP) has the mandate of providing this parenting education. Every month, the LHW and some other front line workers such as school health and nutrition workers, social mobilisers, social welfare workers, CSO workers etc visit each household of his/her catchment area to provide health promotion, and refer to the health center for curative services.



Learning objective:

By the end of the session participants will be able to:

- Describe what parenting means and what is positive parenting.
- Explain the role of parents in positive parenting.



Methodology:

Presentation, role plays, discussions.



Material required:

Flip chart, markers, power point slides, Handouts on positive parenting, Handouts on list of KFCP, nurturing care framework.



Time duration: 2 hours

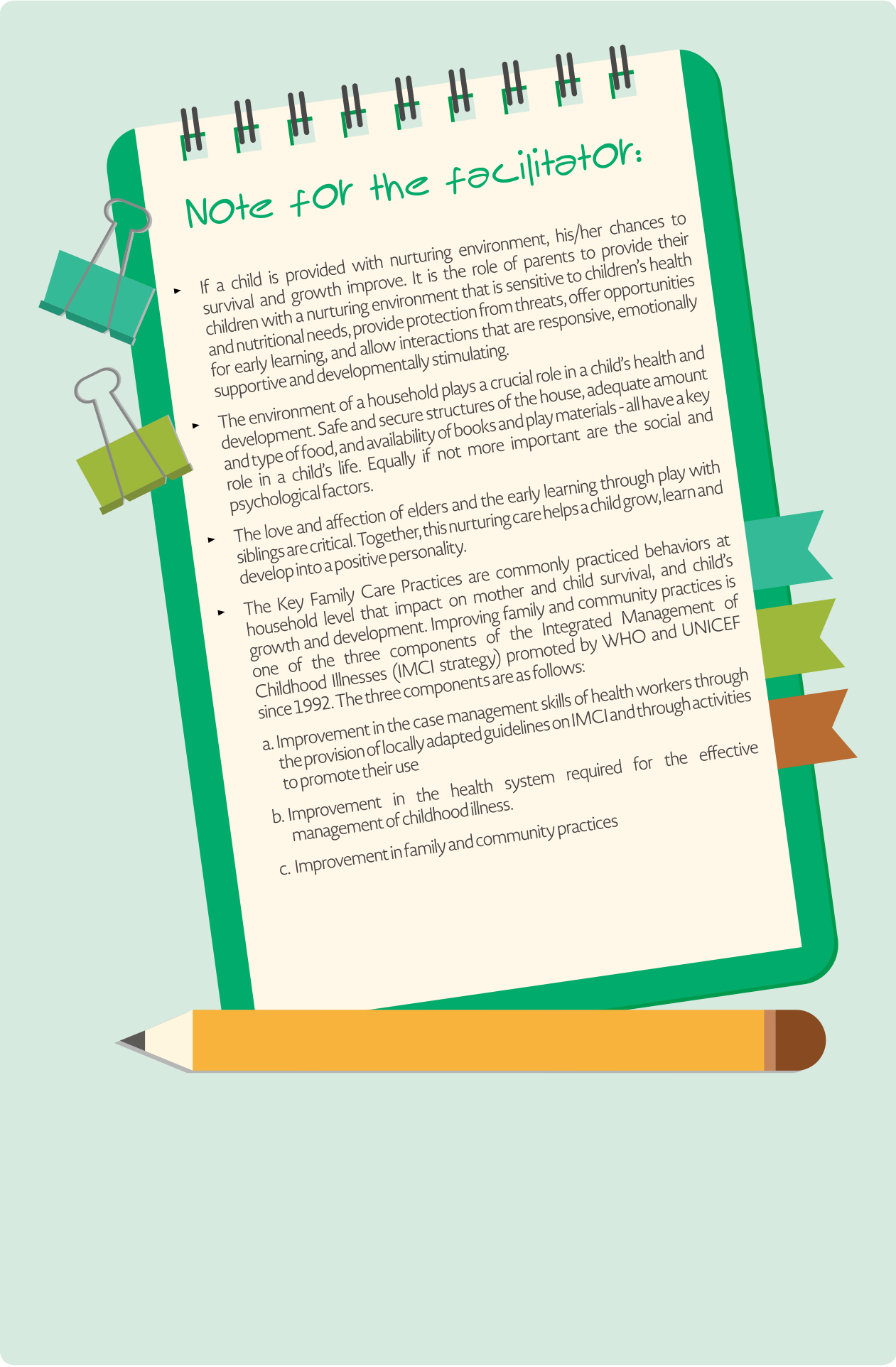
Activity 1: Positive parenting and nurturing and care

1. Divide the participants into three-four groups and ask them to brainstorm on their understanding about:
 - a. what “parents” and positive parenting means.
 - b. what they understand by nurturing and care.
2. Write their responses in the flip chart.
3. Ask the participants if they interact with their children and what are the various occasion in which they interact with children.
4. Do a short presentation on parenting and nurturing and care and its importance.
5. Summarize the key points.



KEY MESSAGES

- Child development is the period from conception (pregnancy) to primary school entry age (0-8 years).
- Parenting can be understood as interactions, behaviours, emotions, knowledge, beliefs, attitudes and practices associated with the provision of nurturing care.
- The term ‘parenting’ or ‘parent’ is not limited to biological parents, but extends to any guardian or caregiver providing consistent care to the child. Those caregivers include fathers, mothers, siblings, grandparents and other relatives, as well as child care providers who play a significant role in caring for infants and young children.
- In some ways, parents determine the life course of a child, as parenting is twice as predictive of a child’s success in early learning as a family’s socioeconomic status.
- When parents interact with children it is not for their children to achieve one outcome, but to develop. Programmes should adapt to this reality of developmental complexity to support nurturing care holistically.



Note for the facilitator:

- ▶ If a child is provided with nurturing environment, his/her chances to survival and growth improve. It is the role of parents to provide their children with a nurturing environment that is sensitive to children's health and nutritional needs, provide protection from threats, offer opportunities for early learning, and allow interactions that are responsive, emotionally supportive and developmentally stimulating.
- ▶ The environment of a household plays a crucial role in a child's health and development. Safe and secure structures of the house, adequate amount and type of food, and availability of books and play materials - all have a key role in a child's life. Equally if not more important are the social and psychological factors.
- ▶ The love and affection of elders and the early learning through play with siblings are critical. Together, this nurturing care helps a child grow, learn and develop into a positive personality.
- ▶ The Key Family Care Practices are commonly practiced behaviors at household level that impact on mother and child survival, and child's growth and development. Improving family and community practices is one of the three components of the Integrated Management of Childhood Illnesses (IMCI strategy) promoted by WHO and UNICEF since 1992. The three components are as follows:
 - a. Improvement in the case management skills of health workers through the provision of locally adapted guidelines on IMCI and through activities to promote their use
 - b. Improvement in the health system required for the effective management of childhood illness.
 - c. Improvement in family and community practices



SESSION 3

THE FAMILY AND HARMONIOUS RELATIONSHIPS

SESSION 3

THE FAMILY AND HARMONIOUS RELATIONSHIPS



Background:

The family is the primary social environment and source of experiences for the child, both because family members or primary caregivers are most in contact with the child. The most salient feature of the family in child rearing is not just the socialization of the child to cultural norms and values, it is also the social and economic resources of the family including parenting skills and education, cultural practices and approaches to intra-familial relations and health status of the family members. Children learn moral values and social roles through a process of socialization which occurs in different domains marked by different aspects of the parent-child relationship. Children learn from the family through riddles, songs, proverbs, myths which reinforce customs and family and community acts like model for expected behaviors. There are several members of the family like mother, father, siblings, grandparents and other relatives who play an important role in child development especially in his sense of security, belongingness, social harmony and group coherence which is a stable and fertile environment for the child to grow. The family supports children as they need to have bonding with at least a family member for a feeling of security.

For a family to function in a healthy manner, there is a need to be effective communication between parents and children. The family need to listen to each other, be open and respect each other, encourage members to express their feelings. Thus qualities of a healthy family relationship are love, trust, honesty, appreciations, patience, caring, mutual respect, endurance, tolerance, protection and safety and help other feel valued and build self-esteem.



Rationale:

Parenting is the process of supporting the physical, emotional, social, and intellectual development of a child from infancy to adulthood. The responsibility for proper child care and development continues to rest with the parents, however there are social protection and other social support systems to assist them in making informed decisions about quality care for their children. The purpose of this session is to examine the different family patterns and relationships within the family and impact child development.



Learning objectives:

On completing this session, participants will be able to:

- Understand role of family and community in early socialization and child development.
- Describe the different family patterns in the Pakistani context and role of members in child rearing especially the role of the father.
- Have the opportunity to share and gain knowledge on how child bonds and forms attachment with others and importance of relationships formation during childhood.
- Understand how healthy relationships in the family enhances child development.



Duration: 2 hours:

Activity 1:

Understand role of family and community in early socialization and child development.

1. Ask participants to draw an external circle leading to family members according to closeness to child and their roles to child care and development.
3. Discuss in the plenary about what is meant by family, the different types of families in the Pakistan/Province context and ask them to describe what they think could be the advantages and disadvantages of each family pattern.
4. Ask them to describe the existing child rearing practices in families.

Activity 2:

Different types of families in Pakistan and their roles

1. Divide the participants into groups and ask them to write down on flip chart,
2. First group: the role of the father in healthy family development. How should father be involved in child care?
3. Second group: the role of the family in child care and other members of the family.
4. Ask the participants to draw a circle with the child in the middle.
5. Who is the most important person in the child's life drawing a line from child-person and why?
6. Discuss especially the role of the father in child care and development.
7. Summarize and conclude.

Activity 3:

Opportunity to share and gain knowledge on how child bonds and forms attachment with others and importance of relationships formation during childhood

1. Divide the participants in four groups and ask each group. Why is it important for the child to grow in a family with harmony and relationship to family and community members?
2. Each group has to write their responses on a flip chart and present later by a group member.
3. Discuss and summarize why a nurturing family environment is beneficial for child development.
4. Explain to the participants that it is not always easy to communicate well as a family. Improving family communication takes practice, and taking time to communicate is even more important than practicing new skills. Ask participants if they have any questions.
5. Ask them to explain how they think they could use this information concerning the family.

Conclude the session.

KEY MESSAGES

1. A family can provide a safe place for its members to share feelings, thoughts, ideas, fears and hopes.
2. Parents should try to find time to talk with each person alone, and together as a family.
3. When you are angry and most worried, it is especially important to plan a few minutes when everyone can be together.
4. A few minutes spent at the end of the day just talking about how things have gone that day and planning for tomorrow can be a relief from stress. Be sure to save difficult problem-solving conversations for times when you are not tired.
5. In Pakistan a lot of children live in extended families especially in rural areas. Grandparents, aunts and uncles as well as older siblings take care of young children.
6. Therefore, it is very important to discuss the role of families in childhood development and care and responsive care giving.
7. The bond children develop with their parents/care givers and families help them to feel secure and protected.





SESSION 4

BUILDING BRAIN IN PAKISTAN AS
FOUNDATION FOR LIFELONG DEVELOPMENT

SESSION 4:

BUILDING BRAIN IN PAKISTAN AS FOUNDATION FOR LIFELONG DEVELOPMENT



Background information

The brain is responsible for human functioning (Movement, thinking, feeling and emotions, perception and how we related to others). Humans cannot function without a good brain. Brain development starts from the moment of conception and grows very rapidly during the first 1000 days of live. In the first 3 years of life the child brain has grown to 80%. The brain is made up of connections that need to be nourished to increase the connections because the more the connections the better the brain. Building Brains means that we are building the intelligence, motor development, social and emotional development which are the main domains of human development.

During the brain development process, the pregnant mother and child need to be healthy, have proper nutrition, be secure and protected, and exposed to learning experiences and STIMULATION because these are like food for the brain. There is compelling evidence now that parents and caregivers can change the brain connections through the sensory experiences (play and communication) with preventing diseases and eating proper food that affects the “wiring or connections” of the developing brain.

Positive parenting helps the child’s brain to grow in a healthy way. The good thing is that this can happen very easily during the period from conception to 3 years of age.



Rationale:

This session will help participants to be aware of how child's brain functions and develops and how the early years are the foundation of lifelong development. The structure of brain is determined by the child’s early experiences and good parent/caregiver-child interactions.



Learning objectives

Upon completion of this session, the participants will be able to:

- Understand the multifunctional nature of the brain and influence on the acquisition of different physical, cognitive, social and emotional skills that determines lifelong development and achievement.
- Understand that parents and caregivers have the power to ensure that children have healthy brains through positive parenting and quality interactions with their children during the early years.



Methodology:

Presentation, role plays, discussions.



Material required:

Flip chart, markers, power point slides, Handouts on ABC.



Duration: 1 hour

Activity 1: (Homework)

1. Tell participants at the end of the first day that they will have an assignment that they will do at home with their children. Share the questions to every participant to take home.
2. Ask participants to identify traditional/local games that parents/caregivers can involve their children of different ages conception, Birth – 3 months, 3-6 months, 6-24 months, 2-3; 4-6; 6-8 years.; Name of play activity and material required for the play activity (Give this as homework so that participants can ask children at home or neighbors).
3. Identify traditional children songs, rhymes and lullabies and at the end of the training catalogue them for community use.

Activity 2: Role play

Divide participants in groups of 5s and ask them come up with plays/games and explain which domain of child development (Cognitive, motor, social and emotional and language competencies) is the play promoting. (The aim of this role play is to show parents that child different domains need to be promoted in different ways because all the developmental domains are intertwined because one cannot function well without the other)

Give them 10 minutes and have a plenary for each group to present their work.



KEY MESSAGES

- Sensitive and critical periods of development impact lifelong development and achievements.
- Children need proper nutrition, good health, positive parenting through stimulation for sensory experiences and protection from abuse, neglect and toxins for the brain to develop.
- Young children need lots of quality interactions to build connections in their brains for learning and healthy development. Healthy environment that stimulate all the senses through singing, story-telling, reading, playing, smelling, tasting, listening, touching, interacting with adults and other children build a healthy brain.
- Musical activities stimulate every part of the brain in all domains: physical, social, emotional, cognitive and language. Listening or singing songs makes the brain healthy and boost the mood of the child.
- The new born brain develops at an astonishing rate and is directly influenced by the quality of relationships and how he is cared in terms of body contact warm and loving care and stimulation of the senses- make child hear sounds, songs, music, see colours etc.
- Talk to the child by repeating and adding other words, and ready to your child every day, tell stories to the child to help simulated the brain. Hum or sing to the child even before he or she is born or listening to soft and melodic songs every day when pregnant. Talk to the child during breastfeeding, feeding while having eye contact.
- Take time to have some contact and affective moments with the child while looking into his/ her eyes- laughing and making the child to smile/laugh.
- Every day a parent should have some interaction at least 30 minutes per day with the child- doing something together that will make the child feel happy.

Session 1:

MODULE 2



SESSION 1

COUNSELLING SKILLS

COUNSELLING SKILLS

COUNSELLING SKILLS



Background:

This manual has used an interactive method of educating and counselling care givers. The LHW/other frontline worker will continue talking about mother and child health issues like before. The difference would be in telling the families that in addition to taking care of their health and nutrition, if parents also provide the opportunities of learning through play, the boys and girls will become healthier and intelligent, and will be economically more productive as an adult.



Rationale:

Often it is thought that providing information about a practice will automatically result in adoption of that practice. Experience tells that it may not always happen. For example, LHW/other frontline worker's message to a mother about 4 antenatal checkups may not always result in the mother going to health center and getting her checkups. From the mother's perspective, there may be one or several valid reasons including having no money to commute, not knowing the procedure, or simply the lack of permission from the family. We know there is a solution for each of these problems but the LHW/other frontline worker will know and facilitate decision making only if she asks about existing situation in the family. The process of knowing the context and providing workable solutions within that context is called counseling.

The role of a LHW/other frontline worker is to facilitate adoption of healthy practices by the family and community. Counseling is the 'way' of doing this. Counseling is helping mother and family in thinking about a specific health practice, reasons why they are not able to fully adopt it, and jointly deciding and implementing the ways for full adoption. This method has been shown very helpful in difficult to change behaviors. While talking about a health and other issues, the LHW/other frontline worker tries to understand the barriers and assist the mother and family in making a decision to overcome that barrier and ultimately adopt the desired behavior. There are three simple steps that LHW/other frontline worker can follow to make her counseling effective. We call them the ABC of communication.



Learning objectives:

By the end of the session, participants will be able to:

- Explain that educating caregivers is beyond just giving information.
- Have improved skills on counselling.
- Practice using the ABC of counseling.



Methodology:

Presentation, role plays, discussions.



Material needed:

Flip chart, markers, power point slides, Handouts on ABC.



Time duration: 2 hours

Activity 1: Understanding of Counselling Cards

- Introduce the concept of ABC of counseling, with the help of slides.
 - Take the participants through the basic skills of ABC of communication: Ask, Brainstorm, Coach:
1. Ask: about the specific practice, whether it is being implemented or not, and reasons of why if not being practiced.
 2. Brainstorm: discuss with mother about solutions of the problem (barrier) that are feasible in the context of her family.
 3. Coach: help the mother in learning the skills involved in that specific practice.
 - Ask them how they educate the families/care givers.
 - Write the responses on a flip chart.
 - Summarize the main points based on the responses.

Activity 2: Role Play

- Divide the participants into four groups.
- Give them a scenario to practice the ABC skills (use the scenario given below).
- Let them do role plays in their topic.
- In the plenary provide feedback to the role plays.
- Summarize the main points of ABC and answer any questions the participants may have.

Role Play:

[Scenario: A pregnant woman who is not going for her antenatal check-ups because her mother-in-law does not agree with that and believes that there is no need. LHW/OTHER FRONTLINE WORKER has to convince the woman and her mother in law with the help of ABC counseling.]

Step 1:

The trainer explains the scenario to the group of participants.

Step 2:

Asks one of the participants to adopt the role of the mother, ask the second to act as a MIL, and third, as a LHW/other frontline worker who has to convince that woman and her MIL.

Step 3:

The trainer explains the participants to use counseling card by focusing the ABC of counseling during the role play. It is important to emphasize that the participant who is playing the role of LHW/other frontline worker should have a positive attitude, show care and encouragement, listen to the parent/caregiver, and fulfill all three steps of the ABC of counseling.

KEY MESSAGES

In this manual, we have introduced a new method of counseling, which is ABC of counseling.

Ask: about the specific practice, whether it is being implemented or not, and reasons of why if not being practiced.

Brainstorm: discuss with mother about solutions of the problem (barrier) that are feasible in the context of her family.

Coach: help the mother in learning the skills involved in that specific practice.

The A of counseling includes:

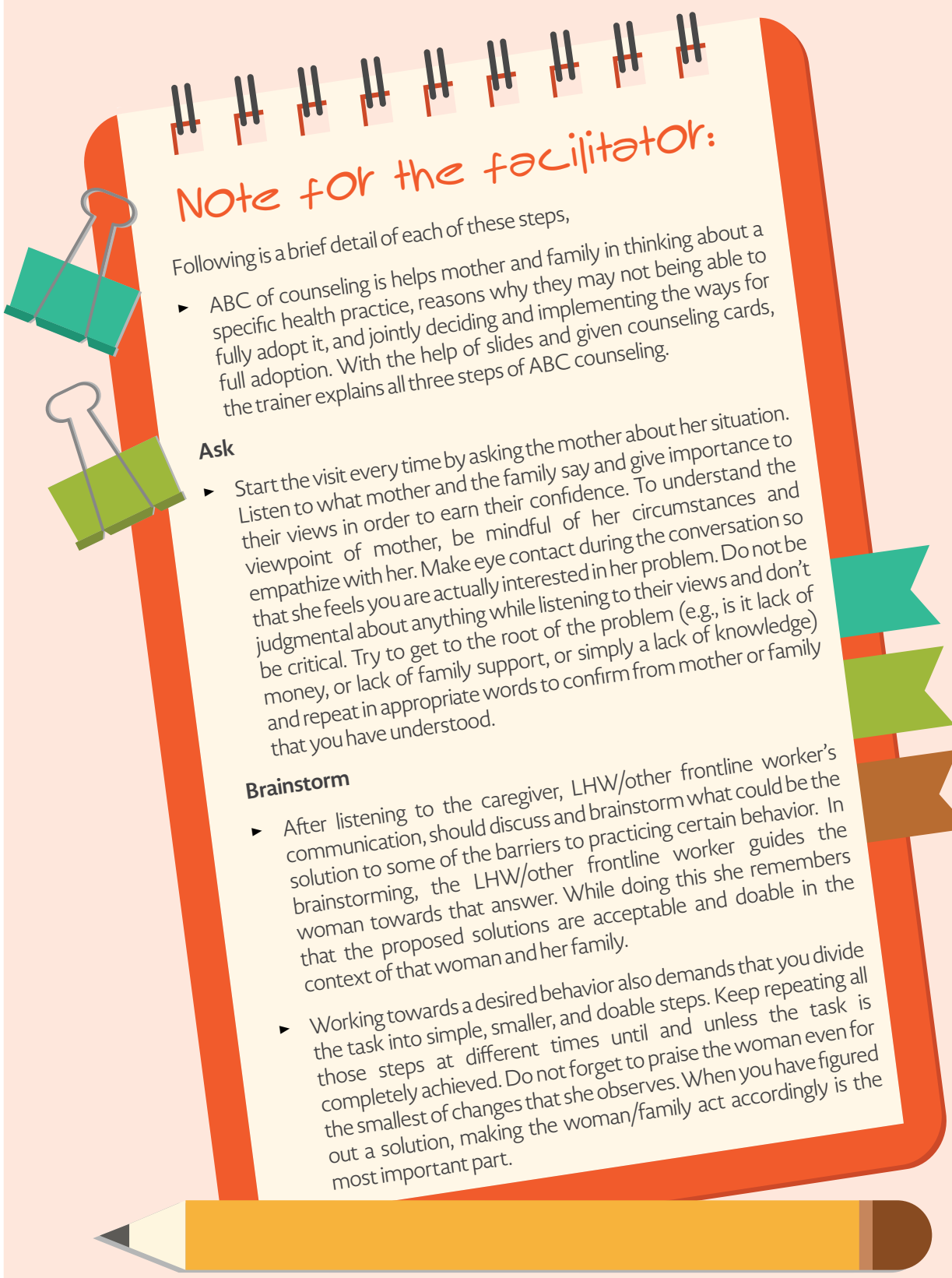
- Start the visit every time by asking the mother about her situation. Develop a friendly relationship.
- Listen to the mother and the family and give importance to their views.
- To understand the viewpoint of mother, be mindful of her circumstances and empathize with her.
- Make eye contact during the conversation so that she feels you are actually interested in her problem.
- Give woman and her family adequate time to speak.
- Do not be instantly judgmental about anything while listening to their views and don't be critical.
- Try to get to the root of the problem.

The B of counseling includes:

- Jointly look for a solution to the problem that was identified in the first (Ask) step.
- Help the woman/family in executing that solution.
- Divide the task into simple, smaller, and doable steps.
- Keep repeating all those steps at different times until and unless the task is completely achieved.
- Do not forget to praise the woman even for the smallest of changes that front-line worker observes.
- When you have figured out a solution, making the woman/family act accordingly is the most important part.

The C of counseling includes:

- Help the mother start implementing the solution that was jointly reached in the 2nd (Brainstorm) step.
- Show patience and ability to demonstrate the new skill to the mother.
- Repeat in appropriate words to confirm from mother or family that you have reached the right conclusion.



Note for the facilitator:

Following is a brief detail of each of these steps,

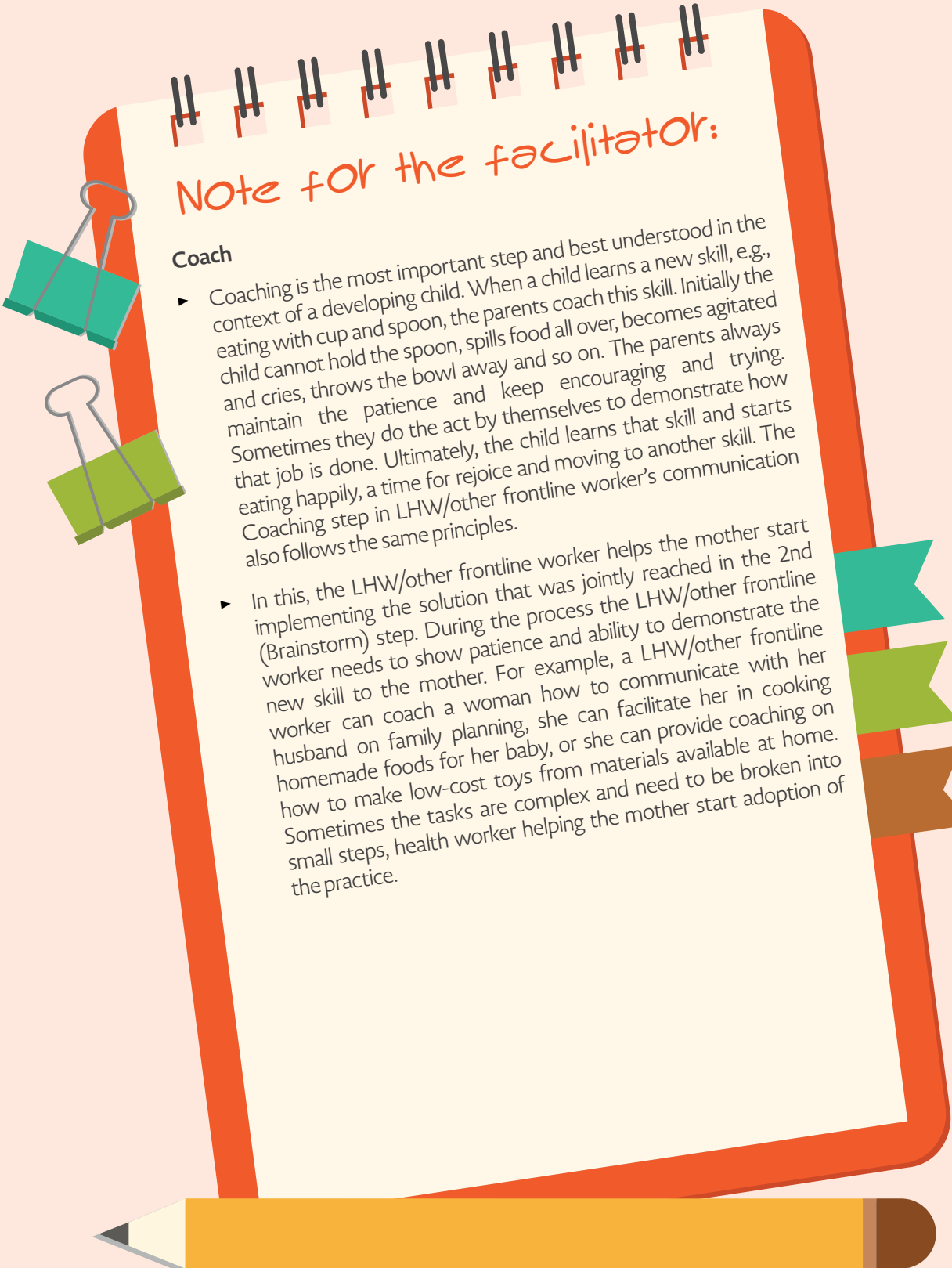
- ▶ ABC of counseling helps mother and family in thinking about a specific health practice, reasons why they may not be able to fully adopt it, and jointly deciding and implementing the ways for full adoption. With the help of slides and given counseling cards, the trainer explains all three steps of ABC counseling.

Ask

- ▶ Start the visit every time by asking the mother about her situation. Listen to what mother and the family say and give importance to their views in order to earn their confidence. To understand the viewpoint of mother, be mindful of her circumstances and empathize with her. Make eye contact during the conversation so that she feels you are actually interested in her problem. Do not be judgmental about anything while listening to their views and don't be critical. Try to get to the root of the problem (e.g., is it lack of money, or lack of family support, or simply a lack of knowledge) and repeat in appropriate words to confirm from mother or family that you have understood.

Brainstorm

- ▶ After listening to the caregiver, LHW/other frontline worker's communication, should discuss and brainstorm what could be the solution to some of the barriers to practicing certain behavior. In brainstorming, the LHW/other frontline worker guides the woman towards that answer. While doing this she remembers that the proposed solutions are acceptable and doable in the context of that woman and her family.
- ▶ Working towards a desired behavior also demands that you divide the task into simple, smaller, and doable steps. Keep repeating all those steps at different times until and unless the task is completely achieved. Do not forget to praise the woman even for the smallest of changes that she observes. When you have figured out a solution, making the woman/family act accordingly is the most important part.



Note for the facilitator:

Coach

- ▶ Coaching is the most important step and best understood in the context of a developing child. When a child learns a new skill, e.g., eating with cup and spoon, the parents coach this skill. Initially the child cannot hold the spoon, spills food all over, becomes agitated and cries, throws the bowl away and so on. The parents always maintain the patience and keep encouraging and trying. Sometimes they do the act by themselves to demonstrate how that job is done. Ultimately, the child learns that skill and starts eating happily, a time for rejoice and moving to another skill. The Coaching step in LHW/other frontline worker's communication also follows the same principles.
- ▶ In this, the LHW/other frontline worker helps the mother start implementing the solution that was jointly reached in the 2nd (Brainstorm) step. During the process the LHW/other frontline worker needs to show patience and ability to demonstrate the new skill to the mother. For example, a LHW/other frontline worker can coach a woman how to communicate with her husband on family planning, she can facilitate her in cooking homemade foods for her baby, or she can provide coaching on how to make low-cost toys from materials available at home. Sometimes the tasks are complex and need to be broken into small steps, health worker helping the mother start adoption of the practice.

Following are the main features of this new way of helping the parents:

1. **Provide information:** Building Brains Pakistan has developed pictorial materials for LHWs/other frontline workers to use for discussion with mothers and other members of the household. These materials contain information about key family care practices in simple language and pictures to illustrate these practices. The information links each practice with the child's health and development. In addition to providing information, the LHW/other frontline worker also asks the family's prevailing practice about it.
 2. **Encourage joint problem solving:** Experience from all over the world informs that providing only the information is not enough for initiating and sustaining healthy practices. Several times the mothers and the families do not adopt desired practices because of certain contextual factors. A health worker cannot facilitate the adoption of desired practice unless she knows about and addresses these barriers in the specific context of that family. LHW/other frontline workers should use the approach of brainstorming for joint solutions to behavioural barriers faced by the care givers.
 3. **Emphasize coaching at all stages:** Discussion, no matter how enriching, is useless if the learning is not put to practice. Building Brains Pakistan ensures this by introducing the element of coaching into the daily work of LHW/other frontline worker as well as the family. We all know that a child who is learning to hold a toy or a spoon for her own feeding fails many times. Parents and other caregivers have to show patience and keep trying till the baby learns that new skill. In the same way, health worker needs to show patience and ability to demonstrate the new skill to the mother. Along the same lines, a trainer has to coach the LHW/other frontline worker.
- The role of LHW/other frontline worker is to facilitate adoption of healthy practices by the family and community. Counseling is the 'way' of doing this. Counseling is helping mother and family in thinking about a specific health practice, reasons why they are not able to fully adopt it, and jointly deciding and implementing the ways for full adoption. This method has been shown very helpful in difficult to change health behaviors.

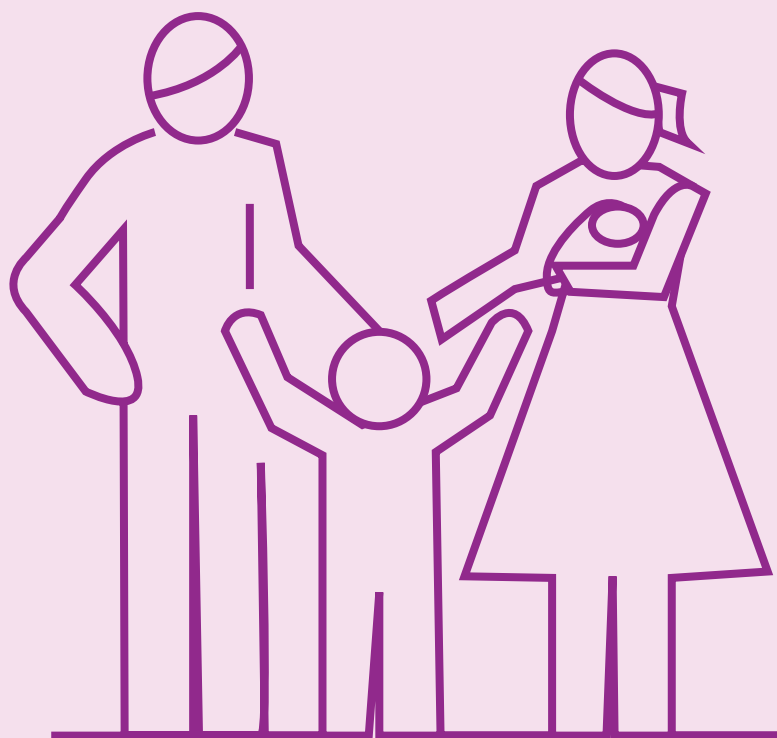
Some practical tips for the frontline workers

1. Decide the time of the visit according to the convenience and agreement of mother/care giver. This would allow the mother to sit in peace with you, listen to you carefully, and be able to inform you better about her problems.
2. Always start the visit with greetings and asking about general wellbeing of the family. This gives a message that you are not here just to fulfill your duty or to give a lecture in a hurry, rather you are actually interested in listening to the family's problems and solving them.
3. If other members of the family are present, greet them as well and if they are interested, make them a part of the conversation. Assure the family that you are actually concerned about the family and mother and child's health.
4. While sitting, make sure that you, mother and rest of the family are sitting at the same level. Frontline workers should not sit on a chair while others are sitting on the floor. If this happens, mother and family members might think themselves as less educated and will not be able to talk to you openly.
5. Start every visit by reminding was discussed and what were the decisions made in the previous visit. If the mother has been able to act accordingly, praise her. Try to understand the reasons if she has remained unable to do those tasks and try to agree with her again to act upon them.
6. Appreciate and praise the mother for taking care of herself and her baby. If you will do this, the mother will listen to any new behavior that you talk about, and try her best to adopt it.
7. Listen carefully what mother has to say and then repeat it in your own words. This makes the mother

feel that you are actually concerned about her problem and are listening to her attentively. This makes her feel contented that the issue she is facing is resolvable. If you are not able to understand her problem, repeating her words will clarify it for you as well.

8. Always make eye contact while talking to mother or listening to her. This will make her feel that you are fully attentive and she will feel that you are actually concerned and sincere towards her. Nodding your head while talking and saying simple motivating words like, "okay", "good" will make the mother continue her conversation easily.
9. When talking to mothers and care givers, always use simple language and easily understandable words. Usually health workers use scientific jargon while talking about simple ailments, which general public cannot understand.
10. Say thank you at the end of every visit. Say thanks to the mother and family for giving you time and listening to what you had to say and ask their permission to leave. Also, decide the time and date for your next visit with the family.

MODULE 3



KEY FAMILY CARE PRACTICES

SESSION 1:

Key Family Care Practices:

Global health leaders like the WHO and UNICEF have developed a set of household practices that are key for a healthy mother and child. Adoption of these practices significantly contributes to mother and child survival development and protection, paving the way for a healthy, happy and prosperous family and society. Countries have adopted as well as adapted these practices according to their context. The present list of KFCPs for Pakistan incorporates the preexisting resources; both international and national, and is finalized after consultation with experts from all provinces and regions. This list of 22 KFCPs for Pakistan adopts a life course approach and addresses health and development needs of the mother-infant dyad through various stages of life. Promotion of these practices is also a means of ensuring inter-sectoral collaboration as these address diverse areas including health, WASH, child protection, nutrition, child development, and child education. The material with list of key family care practices should be left at the household which the parents and care givers can refer to.

Counseling Cards:

Counseling cards for LHW/other frontline worker are based on Key Family Care Practices (KFCPs). One card addresses one KFCP and targets specific stage during the life course. Broadly the stages addressed are 1) Pregnancy and childbirth 2) Newborn period 3) Infancy 4) Toddler age 5) Preschool age 6) Primary school age. Each card has pictures for LHW/other frontline worker to use for generating discussion using the ABC (Ask, Brainstorm, Coach) of communication approach.



Methodology:

Presentation, role plays, discussions.



Material needed:

Flip chart, markers, power point slides, Handouts on ABC.



Time duration: 2 hours

Activity 1: Key Family Care Practices

1. Ask the participants what are the key practices they follow to take care of their 0-8 years children.
2. Write their responses in the flip chart.
3. Share the handout with the list of 22 key family care practices.
4. Have a discussion on how these practices covers the life course approach and its importance.

Activity 2: Introduction to the counselling cards

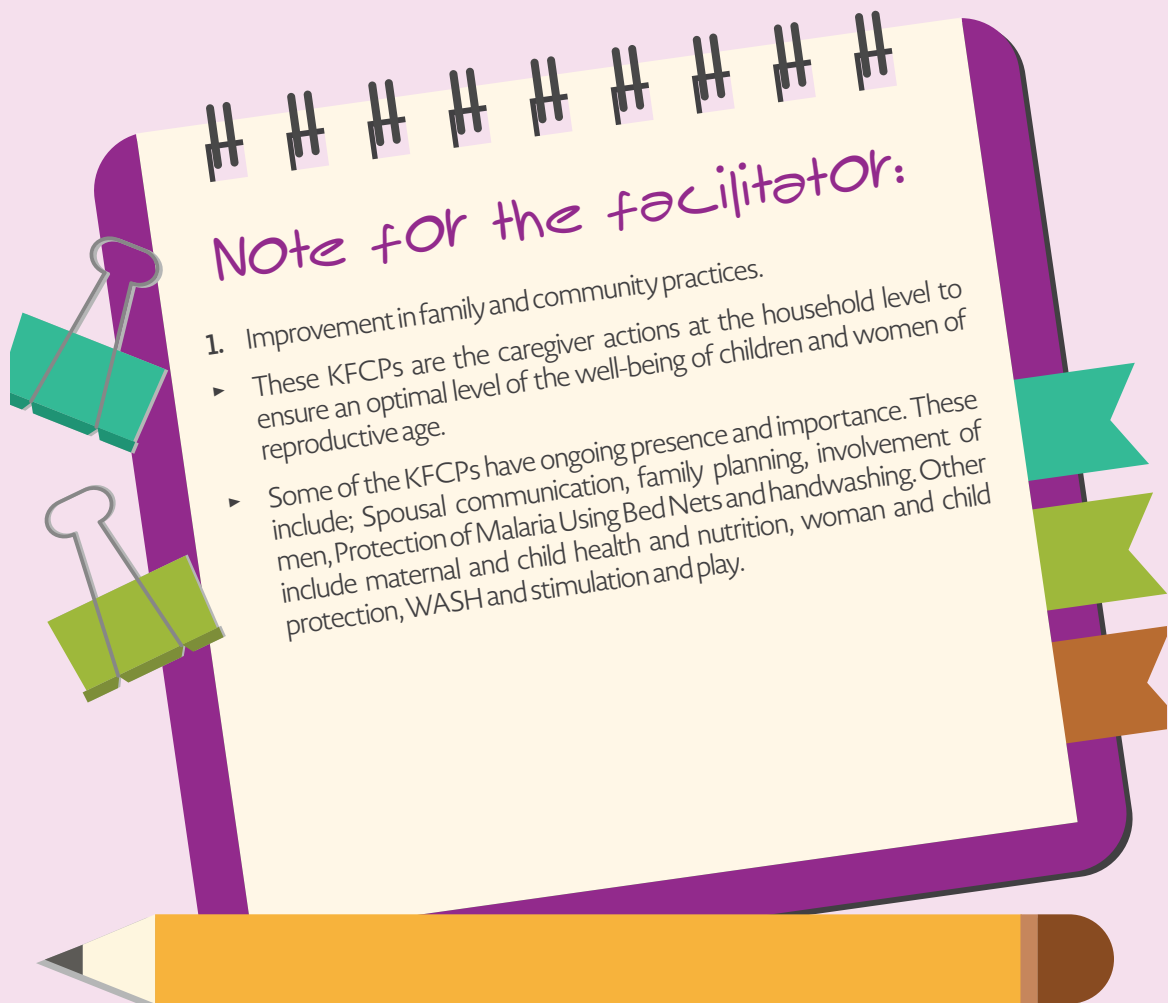
1. Share the counselling cards with the participants.

2. Have a discussion on how these counselling cards can be used.
3. Do a demonstration on how to use the counseling card to educate families.
4. Explain that in the subsequent session, participants will be able to practice using the cards according to the family care practices.

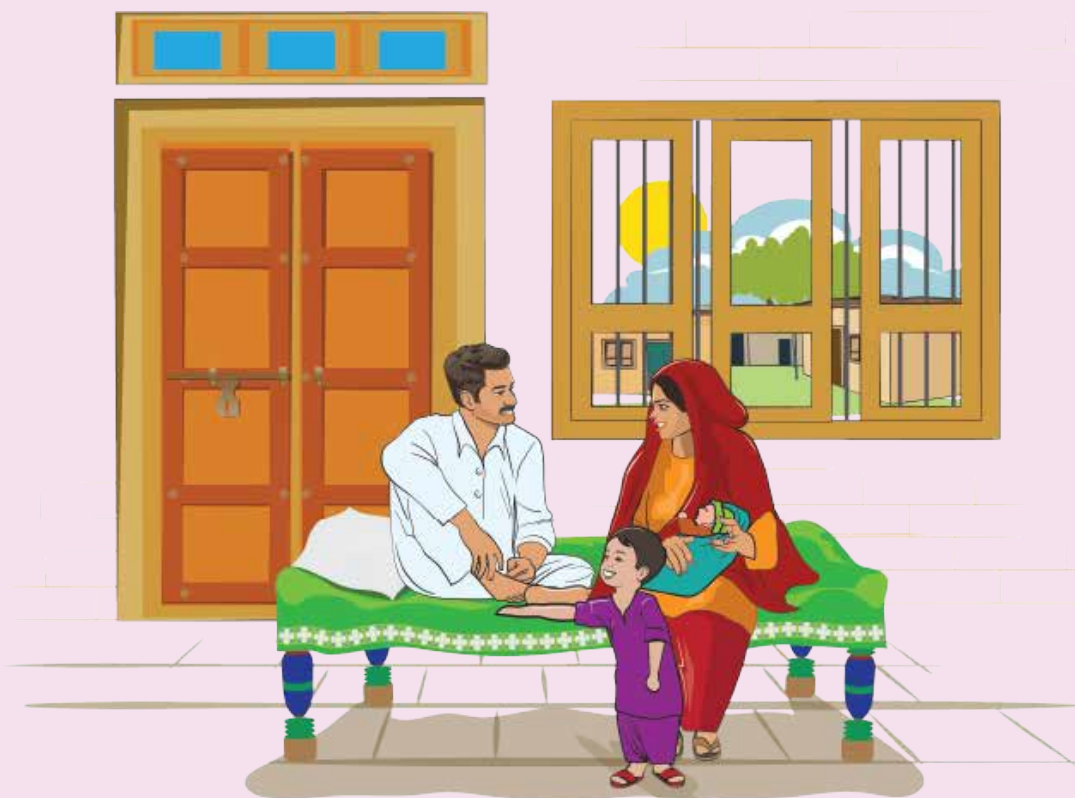


KEY MESSAGES

- The key family care practices are commonly practiced behaviors at household level that, if practiced properly, impact a child's survival, growth and development. These key practices are added in the manual after a lot of consideration and talks about many important practices that have not been discussed before.
- These practices have been selected because they are by far the most likely to cause an impact on nutrition, survival, growth, wellness and development. The below list has been developed by adapting and collating similar lists proposed in Pakistan and in other countries.



Practice 1



SPOUSAL COMMUNICATION

PRACTICE 1:

Husband and wife should talk to each other frequently about family matters including child's and mother's health, growth and development; and positive discipline



Learning objectives:

By the end of the session, participants will be able describe the importance of spousal communication.



Methodology:

Presentation, role plays, discussions.



Material needed:

power point slide, handouts, flip chart and markers.



Time duration: 1 hours

Activity 1:

1. Greet the participants and requests a few participants from the group to summarize the key points discussed in previous session. He/she appreciates the participants (if they remembered important points) and repeats key points.
2. Ask the participants about the role of communication among spouses and why is it important?
3. Write down important points on a flip chart and tell them that today we will talk about the importance of effective communication between partners/couples.
4. Discuss with help of the slides the key points.

SLIDES

Slide 1: Understanding each other's needs

- key to healthy relationship
- Strengthens the husband-wife bond
- Effective planning for nurturing environment

Slide 2: Communication defines roles and responsibilities

- Equal participation of partners
- Birth preparedness
- Antenatal and postnatal care
- Child rearing
- Birth spacing

Slide 3: Communication gap and reasons

- Lack of awareness
- Cultural norms
- Shyness/ Taboo
- Husband does not have the time

5. Now ask the participants what is their common observation? Do wives discuss family related matters with their husbands in our communities? If they say no, ask them why?
6. Listen to the reasons carefully. If they mention cultural norms, shyness, or time constraints, ask

them how can we solve these problems? Help them brainstorm possible solutions of these barriers.

7. Write down these barriers and their solutions on a flip chart.



KEY MESSAGES

- Spousal communication is a gateway behavior. It helps partners understand each other's' needs; both physical and mental, and ensures a loving and healthy environment.
- Spouses who discuss everything on a daily basis can;
 - plan their family better.
 - take better care of each other's needs.
 - space their children.
 - take better care of health, development and education of their child.
 - provide better opportunities to their child.
 - Poor communication causes conflicts in relationships and can threaten the existence of the relationship. Couples who do not discuss about daily matters lack understanding and bonding, that not only affect them but their children as well.

Practice 2



INTERVAL/ SPACING BETWEEN BIRTHS

PRACTICE 2:

Avoid unplanned pregnancy and keep space of 3 years between birth of children with mutual consensus:



Learning objectives:

By the end of the session,

- Participants will be able to describe the importance of birth spacing and benefits to the woman, child and family.
- Will be able to have knowledge about various family planning methods available.



Methodology:

Presentation, role plays, discussions.



Material needed:

power point slide, handouts, flip chart and markers.



Time duration: 1 hour

Activity 1:

1. Introduce the next topic: birth spacing.
2. Discuss the importance and benefits of birth spacing and misconceptions attached to it.
3. Ask the participants about their understanding of the word "birth spacing/family planning"? Do they know about its benefits?
4. Listen to them attentively and note down important points on a flip chart.
5. Ask the participants what is their common observation, do people use contraceptive methods? What are most used methods? Are there any misconceptions attached to birth spacing and what is community's attitude towards it?
6. Highlight important points. With the help of slides, discuss the topic in detail and inform them about the importance of birth spacing, various contraceptive methods and misconceptions attached to it.

SLIDES

Slide 1: Concept of birth spacing

- Why children need to be spaced?
- Ideal birth interval
- Birth Spacing and suitable environment

Slide 2: Misconceptions/ barriers regarding family planning

- Western conspiracy
- Husband's lack of interest
- Communication gap & shyness
- Unavailability of contraceptives
- Previous experiences

Slide 3: How to resolve the barriers

- Talk to an expert/LHW/Frontline worker
- Use of appropriate contraceptive methods
- Spousal communication
- Acknowledging the need

Slide 4: Contraceptive methods & their side effects

- | | |
|-----------------|----------------|
| • Pills | Condoms |
| • IUDs | Implants |
| • Sterilization | Tubal ligation |
| • Vasectomy | |

Slide 5: Role of LHW/OTHER FRONTLINE WORKER

- Be a friend
- Facilitate
- Generate demand

Activity 2:

Explains each method of family planning one by one.

Various methods of contraception:

- **Pills:** These contain different combinations of synthetic estrogens that interfere with ovulation. The woman is supposed to take one pill a day, preferably at the same time.
- **Condoms:** Condoms protect against pregnancy and sexually transmitted diseases. They prevent direct contact between the male and female sexual organs.
- **Implants:** An implant is inserted in the arm and can last as long as three months, but it should be removed after that.
- **Sterilization:** This is a permanent method and cannot be reversed. In this method, fallopian tubes are cut and therefore cannot link the ovaries and uterus any more.
- **Tubal ligation:** It is another surgical procedure that involves cutting of fallopian tubes. This is another irreversible method.
- **Vasectomy:** This is surgical procedure for males. It is also irreversible and involves cutting and closing the part between the testes and urethra.

Now talk about the misconceptions that are attached to birth spacing and lead the participants to brainstorm answers for that. Listen to their points carefully and then add your points.

Activity 3:

After this exercise, prepare the classroom for a role-play.
Conclude the session with key points

Role Play:

[Scenario: A family (mother-in-law and woman) who thinks talking to husband about sensitive matters is embarrassing and thinks that woman should talk to her mother in law instead, regarding her needs. Mother in law is authoritative and her daughter-in-law is shy.]

Step 1:

The trainer should explain the scenario to the group of participants.

Step 2:

Ask one of the participants to adopt the role of a mother in law (MIL), second should act as Daughter in law (DIL) and the third participant should act as a LHW/other frontline worker who has to convince that MIL and DIL.

Step 3:

The trainer should explain the participants to use counseling card during the role play. It is important to emphasize that the participant who is playing the role of LHW/other frontline worker should have a positive attitude, show care and encouragement, listen to the parent/ caregiver, and fulfil all three steps of counseling.



KEY MESSAGES

- Birth spacing means interval between two child births. A woman should wait for at least 3 years for her next pregnancy. If she does not wait, she can face a lot of complications like, low birth weight baby, hemorrhage or birth related complications.
- Birth spacing is important because it gives time to the mother to heal properly and recover from her previous delivery.
- A mother with proper birth spacing can give her children and house proper attention which leads to a positive and healthy home environment.
- It also allows the family to plan their resources so that they are able to get the best out of available resources.
- Men should be involved in reproductive health and family planning.

Practice 3



ANTENATAL CARE

PRACTICE 3:

Pregnant women should take tetanus toxoid vaccine and should take at least four antenatal visits to a qualified doctor or health worker before delivery.



Learning objectives:

By the end of the session,

- Participants will be able describe the key components of antenatal care.
- Will be able to have knowledge about birth preparedness and complication readiness.



Methodology:

Presentation, role plays, discussions.



Material needed:

power point slide, handouts, flip chart and markers.



Time duration: 1 hour

Activity 1:

1. Ask the participants if they know how many times a pregnant woman should go for antenatal check-ups and what happens during the antenatal care (ANC).
2. Ask participants about the barriers. Why do women don't go for antenatal checkups.
3. Add your points and write down these barriers on a flip chart. Now ask the participants to brainstorm for possible solutions. Provide facilitation where required.
4. Ask them what are the things that a pregnant woman and families should do to prepare for the birth of the new baby.
5. Write their responses on the flip chart.
6. Do a power point presentation on antenatal care using the slides given below.

SLIDES

Slide 1: Importance of Antenatal Visits

- Keeps a check on pregnancy
- Provides a regular update
- Informs about body requirements & monitors fetal growth

Slide 2: Timings of antenatal checkups

- 1st checkup: as soon as woman gets pregnant
- 2nd checkup: between 4-6 months or pregnancy
- 3rd & 4th checkup: between 7-9 months

Slide 3: Reasons of not receiving antenatal visits

- Lack of awareness
- Lack of resources
- No nearby Hospital/clinics
- Cultural beliefs/ TBAs
- No support from husband/family

Slide 4: Knowledge regarding checkup details

- Confirmation of pregnancy
- Height & weight
- Blood pressure examination
- Blood and urine test
- Ultrasound
- Adequate nutrition & provision of Iron folate
- Deworming
- Date of next visit
- counseling on obstetric danger signs and birth preparedness and complication readiness

Slides 5: Picture: Overcome the barriers

- Talk to the mother/family
- Discuss the importance
- Provide guidance

Activity 2:

1. Make them do a role play on how the LHW/frontline worker would educate a pregnant woman and her family.
2. Summarize key points and answer any questions the participants may have.

Role Play 1:

[Scenario1: A careless family; husband thinks such matters are womanly matters and men have nothing to do with it, a MIL who thinks that in her times there were no antenatal checkups and this is nothing but a way of stealing poor people's money, and a woman who is pregnant and helpless.]

Step 1:

The trainer should explain the scenario to the group of participants.

Step 2:

Ask one of the participants to adopt a role of LHW/other frontline worker whereas the other participants should play the role of a husband, MIL and pregnant woman, respectively.

Step 3:

The trainer should tell the participants to use counseling card during the role play. It is important to emphasize that the participant who is playing the role of LHW/other frontline worker should have a positive attitude, show care and encouragement, listen to the parent/caregiver, and fulfil all three steps of the ABC of counseling.

Discussion:

After role play, the trainer should appreciate the effort of the participants who did the role play. The trainer first asks them about their performance, especially the one who acted as a LHW/other frontline worker. Listen to their views carefully and then ask other participants

about their views. Write down all these points on a flip chart. Explain to the participants about the things that they did well and things that they could do better. Conclude with key points of the session.



KEY MESSAGES

- Have a pregnancy test as soon as you miss one (1) menstrual period.
- Avail 4 check-ups spread over entire pregnancy, from a skilled provider
- Ask the health worker about when to come for ANC.
- ANC check-up will include:
 - Blood pressure examination
 - Blood and urine test
 - Provision of Iron/folic acid
 - Tetanus Toxoid (TT) injections
 - Health education on danger signs, nutrition, date of next visit, birth planning etc.
- Prepare for the birth of the child beforehand: save money, arrange for transport, find out people who can donate blood etc.
- Know about danger signs in pregnancy: any bleeding, fever, swollen hands and feet, blurred vision and fits
- Sleep under insecticide-impregnated bed net
- Sing and talk to your baby while he/she is still in your womb

NOTES FOR THE FACILITATOR:

- Antenatal care is preventive health care. It provides regular checkups and guidance for pregnant women in order to identify and avoid complications during pregnancy.
- A pregnant woman is advised to have at least 4 antenatal checkups. These checkups allow women to monitor fetal growth, get tetanus immunization, receive iron tablets and get advised for nutrition and safe delivery.
- It is recommended to consult only a trained skilled birth attendant. Seek an SBA if following danger signs appear;
 - Bleeding spotting or watery discharge.
 - Severe headaches or blurry vision.
 - Fits.
 - Swelling.
 - Lower abdominal pain.
- The pregnant woman and the family should plan and prepare for the delivery of the newborn baby.
 - Save money for delivery, transportation or other costs.
 - Organize for a transportation beforehand.
 - Identify people who can donate blood if needed.
 - Know the danger signs and when and where to seek care in case of danger signs.

Practice 4



SKILLED BIRTH ATTENDANCE

PRACTICE 4:

Consult nearest hospital or qualified doctor during pregnancy and maternity to avoid complications.



Learning objectives:

By the end of the session,

- Participants will be able describe the importance of having skilled birth attendant at birth.
- Will be able to understand where to go to for skilled birth.



Methodology:

Presentation, role plays, discussions.



Material needed:

power point slide, handouts, flip chart and markers.



Time duration:

30 mins

Activity 1:

1. Ask the participants if they know where do women usually go for delivering babies.
2. Ask them if they know what is meant by skilled birth attendant and if they know who the skilled birth attendant is in their community.
3. Ask participants about the barriers. Why do women don't go for institutional delivery?
4. Ask them if women go for postnatal check- ups and where.
5. Add your points and write down these barriers on a flip chart.
6. Now ask the participants to brainstorm for possible solutions. Provide facilitation where required.
7. Write their responses on the flip chart.
8. Do a power point presentation on SBA using the slides given below.

SLIDES

Slide 1: Consulting an Skill Birth Attendance (SBA)

- Most mother-child deaths occur at the time of delivery
- SBA can prevent life threatening complications
- Identify who the SBA is in your area

Slide 2: Barriers

- Family tradition/ Traditional birth attendance (TBA)
- Lack of money
- 3 delays

Slide 3: Postnatal checkup

- Informs about physical health of mother
- Should be carried out within 6 hours after delivery
- Identifies dangers

Slide 5: Women don't get postnatal checkups because;

- They think it is unnecessary
- Lack of knowledge
- Newborn requires more attention
- Woman is out of danger now

Slide 4: Postnatal danger signs

- Excessive bleeding
- High fever
- Fits
- Foul discharge



KEY MESSAGES

- It is important to know who the skilled birth attendant is in your community.
- Couple should go together to the health facility for regular check-ups when the woman is pregnant.
- Skilled birth attendant can: attend delivery safely, recognize danger signs, advise the woman and her family about what actions to take in case of danger signs; manage and refer complications to appropriate health facility.

NOTES FOR THE FACILITATOR:

- Most of the maternal and newborn deaths occur at the time of delivery. A major reason behind these deaths is not consulting a skilled birth attendant in pregnancy and at the time of child birth.
- A skilled birth attendant is a person who provides essential and emergency health-care services to women and their newborns during pregnancy, childbirth and the postpartum period. This include: Lady health visitor, community midwife, nurse and doctor.
- By keeping an eye on danger signs and timely consulting an SBA can protect mother and her baby form expected complications.
- To ensure safe delivery, the woman and family particularly the husband should ensure three things before time. These are:
 - Selection of the place of delivery.
 - Transport.
 - Money.
- The role of family is crucial at the time of delivery. Monitor the health of the pregnant woman and inform the SBA if any danger signs appear.
- It is advised that women should get their postnatal checkup within 6 hours after child birth.

Practice 5



BREASTFEEDING

PRACTICE 5:

Start breastfeeding the baby immediately (within an hour) after birth of child and exclusively breastfeed the newborn for six months.



Learning objectives:

By the end of the session,

- Participants will be able describe the importance early initiation of breastfeeding.
- They will be able to understand what exclusive breastfeeding means.



Methodology:

Presentation, role plays, discussions.



Material needed:

power point slide, handouts, flip chart and markers.



Time duration: 30 mins

Activity 1:

1. Divide the participants into 4 groups and ask them to brainstorm on:
 - What is the best nutrition for the baby immediately after birth?
 - When should the mother start breastfeeding?
 - Do mothers feed colostrum to their newborns in the community. If not what are the reasons?
 - Do they understand the concept of early Initiation and exclusive breastfeeding?
 - What is their common observation regarding exclusive breast feeding?
2. Ask them to write their responses on a flip chart.
3. Using the slide below present what is meant by early initiation and exclusive breastfeeding.

SLIDES

Slide 1: Early Initiation

- Provides necessary warmth
- Develops mother-newborn bond and responsive care to the child
- Initiates early milk let down
- Colostrum - Nutrient rich first feed that the child should not miss
- Makes breastfeeding easier

Slide 2: Barriers in early initiation

- Cultural practices/traditional norms
- Mothers and families perceive that the mother does not produce enough milk
- Cesarean section
- No family support
- Improper way of breastfeeding

- Increases milk let down - Promotes healthy weight and boosts mental development	
Slide 3: Guidance form LHW/other frontline worker - Guide mother/ family - Ensure family support - Select a family member as a helper before time - Guide on proper technique of breastfeeding	Slide 4: Significance of Exclusive Breastfeeding - Only mother's milk for 6 months - No other liquid, not even water - Feeder/Cow milk is a NO! - Good for both mother and her child. - Provides all necessary nutrients - Cost-effective and good the economy of the family
Slide 5: Barriers/misconceptions - First baby - Insufficient milk - Water is a must - Pregnancy - Family influence - Overburdened mother - Working mothers	

Activity 2:

1. After brainstorming, prepare the class for role-plays. The trainer can conduct either one or both role-plays, depending upon the time and situation.
2. After role play, appreciate the effort of the 3 participants who did the role play.
3. Ask the participants who participated in the role-play about their performance and then ask other participants about their views.
4. Listen to their points carefully and note down important points on a flip chart.
5. Explain to the participants about the things that they did well and things that they could do better.
6. Sum up the role play with important points and answer any questions the participants may have.

Role Play:

[Scenario: A weak mother who is overburdened with household work and wants to put her child on formula milk because she does not have time and she also thinks that her breast milk is insufficient for her child]

Step 1:

The trainer should explain the scenario to the group of participants.

Step 2:

The trainer then select 3 participants form the group. Ask one of the participants to adopt the role of woman, second participant should act as a mother-in-law, whereas the third participant should play the role of a LHW/other frontline worker who has to convince that MIL and her daughter in law about early initiation of breastfeeding and exclusive breastfeeding.

Step 3:

The trainer should explain the participants to use counseling card during the role play. It is important to emphasize that the participant who is playing the role of LHW/other frontline worker should have a positive attitude, show care and encouragement, listen to the parent/ caregiver, and fulfil all three steps of the ABC of counseling.



KEY MESSAGES

- Give colostrum (mother's first milk) to your newborn as a first feed within one hour after delivery because it is nutritious and good for the baby.
- Breastfeed the baby on demand that is any time the baby needs to suck even at night.
- Select a support person before time and ensure she helps the woman in breastfeeding her baby.
- Continue mother's milk and nothing else -not even water-for six months.
- Mothers milk protects babies from diseases.
- Eat well and drink lots of water so that the breastmilk can flow well to satisfy the child.
- When breastfeeding the baby, look at the baby in the eyes and smile at him/her, talk or sing to him/her. This will help in their cognitive development and establish a good relationship with your child.
- Provide ways for your child to see, hear, feel, smell, move freely, and touch you while breastfeeding.
- Slowly move colourful things for your child to see and reach for. Sample toys: shaker rattle, big ring on a string.

NOTES FOR THE FACILITATOR:

- Provision of breast feeding to the baby within first hour after birth is called early initiation and has many benefits for both mother and child.
- "Colostrum" the first milk is nutrient rich and has all essential protecting factors that are required for a baby after birth.
- Exclusive breastfeeding means feeding a baby for first six months entirely on breastmilk without giving anything else. The more a mother will breastfeed her baby, the more will be the production of milk.
- Breastfeeding immediately after birth also ensures skin to skin contact between mother and her child which helps in initiating breastmilk early and increases the likelihood of exclusive breastfeeding.
- Exclusive breastfeeding develops a close bond between mother and her child. It is also a natural method of birth spacing.
- Breastfeeding protects against diarrhea and common childhood illnesses, such as pneumonia, and have long term benefits as well.
- For mothers, early initiation of breastfeeding immediately after delivery helps the uterus to contract and prevents woman from excessive bleeding post-delivery.
- Family's role becomes crucial at this point. A family should identify a support person before time, who would assist the mother in breastfeeding her child.
- Coaching new mothers on how to breastfeed their babies and express breast milk as a means of maintaining lactation should be provided to expected mothers.
- Mother's milk is a complete diet for the baby and the benefits of mother's milk are lost if the child is given additional items like water, honey, rose water, cow or formula milk, tea, or other fluids.
- Breastmilk is easily absorbed as a child's digestive system is not fully developed in the first 6 months.
- Breastmilk is hygienic, pure and easily available.

Practice 6



ESSENTIAL NEW BORN CARE

PRACTICE 6:

Provide warmth to the newborn, keep the umbilical cord clean and dry and keep applying chlorohexadin on it once daily for a week. And avoid bathing the newborn within first 24 hours after birth.

Learning objectives:

By the end of the session,

- Participants will be able explain about how to take care of newborn babies at home.



Material needed:

power point slides, handouts, flip chart and markers.



Methodology:

presentation, role plays, discussions.



Time duration: 30 mins

Activity 1:

1. Ask the participants how newborn babies are looked after at home and who looks after them
2. Ask questions like; when a child should be given a bath, what to be used on umbilical cord? Write their responses in the flip chart.
3. Present key ideas on newborn care based on the slides below.
4. Summarize key points and ask any questions they may have.

SLIDES

Slide 1: Wrapping and delayed bathing

- Protects from hypothermia
- Stables baby's body temperature

Slide 2: General perceptions

- Newborn is dirty and needs to be cleaned
- Cultural/ religious practices

Slide 3: Safety measures for umbilical cord

- Application of medicine recommended by the doctor
- Cleanliness of the cord and its surrounding area
- Avoid Cow dung/ghee/ kajal or any other local remedy

Slide 4: Provide guidance

- Talk to the mother/family
- Demonstrate proper application of medicine

Activity 2:

1. Now demonstrate the proper way of applying medicine on baby's umbilical cord and its surrounding areas. Make use of a dummy or doll. [If possible, divide the participants into 4 groups and ask them to practice this activity on their dummy.]
2. After the activity, ask the participants about common barriers attached to essential newborn care.
3. Ask the participants why do people avoid Essential Newborn Care (ENC) and newborn checkups?
4. If they mention carelessness, lack of awareness or cultural beliefs, appreciate them.
5. Ask them what are common perceptions and practices regarding bathing and safety of umbilical cord? What do people normally apply on umbilical cord?
6. If they mention chlorhexidine, appreciate them.
7. If they mention soil, ghee or cow dung, listen to them carefully and write down all the problems on a flipchart. Now help them brainstorm and find the possible solutions for all these barriers.
8. After this exercise, prepare the class for a role play.



KEY MESSAGES

- Essential newborn care ENC is recommended for every newborn. Essential Newborn Care (ENC) should be given immediately after birth and continued till first 7 days after child's birth. This essential care can be provided by any SBA.
 - It is advised to put child to mother's chest immediately after birth and should not be bathed for at least 24 hours. The baby should be dried immediately and covered with a clean and dry towel or cloth.
 - Many newborn deaths occur due to breathing problems. It is advised for the family/ SBA to rub baby's back and feet immediately after birth in order to stimulate crying.
 - Newborns are more prone to diseases and infections. Therefore, newborns should be kept clean. Mother/family should wash hands before touching the baby. Umbilical cord and its surrounding area should be kept cleaned and dry and only prescribed medicine should be applied on it.
 - Family is advised to assist the mother in early initiation of breastfeeding. It is advised to breastfeed the baby within one hour after birth. Colostrum should be given as it contains all the nutrients that a newborn requires.

Practice 7



DANGER SIGNS IN NEW BORN

PRACTICE 7:

Please consult doctor or health worker, if the child is refusing breast feed, is turning pale or blue, is having difficulty breathing or is lethargic.

Learning objectives:

By the end of the session,

- Recognize the danger signs among newborn babies.



Material needed:

power point slides, handouts, flip chart and markers.



Methodology:

presentation, role plays, discussions.

Activity 1:

1. Ask them if they recognize what are the danger signs that parents/care givers should look out for.
2. Emphasize that right after birth, it is important for mother and family to keep an eye on newborn danger Signs, keep newborn warm and do recommended safety measures for umbilical cord.
3. Summarize key points and ask any questions they may have.

SLIDES

Slide 1: Newborn checkup

- Identifies possible dangers
- Consult and SBA
- Responsibility of the family

Slide 2: Possible Danger Signs

- Not feeding properly
- Skin colour turning blue/purple
- convulsion
- Premature birth (before 35th week)
- Trouble breathing
- Very low or very high body temperature

Slide 3: Possible barriers

- This is SBA's job not ours
- Cultural/religious beliefs
- Mother is the one in danger

Slide 4: Role of LHW/other frontline worker

- Talk to the mother/family
- Guide the family on danger signs

KEY MESSAGES

- It is the responsibility of parents/family to look for possible danger signs and consult a doctor immediately if a sign appears. These danger signs are:
 - Not feeding properly.
 - Skin colour turning blue/purple.
 - convulsion.
 - Premature birth. (before 35th week)
 - Trouble breathing.
 - Very low or very high body temperature.
 - For a newborn, the most crucial time is right after birth. By taken safety measures during this period, newborns can be protected against possible diseases and complications.

Practice 8



BIRTH REGISTRATION

PRACTICE 8:

Register birth of boys and girls as soon as possible and obtain a birth certificate



Learning objectives:

By the end of the session,

- Participants will be explain the importance of birth registration.



Material needed:

power point slide, handouts, flip chart and markers.



Methodology:

presentation, role plays, discussions.

Activity 1:

1. Ask the participants if they have registered the birth of their sons and daughters.
2. Ask them why most of the people don't get their children registered? What could be the possible reasons behind this?
3. Listen to the participants and write their main points on a flip chart.
4. Ask them to brainstorm for possible solutions and write down these solutions on the flip chart. Now prepare the class room for a role play.
5. With the help of the slides below explain the importance of birth registration and where they can go to register birth of their children.
6. Begin the discussion by telling them that birth certification is a legal requirement and parents/family should obtain birth certificates so that their sons and daughters are registered for various rights and facilities.

SLIDES

Slide 1: Importance of birth certification

- What is birth registration
- How it is done
- Where to apply?
- where should care givers for birth registration?

Slide 2: Challenges to birth certification

- Lack of awareness
- Birth at home
- Cultural norms

Activity 2:

1. Ask the participants to do a roleplay as per the scenario below.
2. After role play, appreciate the effort of the three participants who did the role play.
3. The trainer then ask other participants about their views.

4. Listen to their points carefully and note down important points on a flip chart.
5. Explain to the participants about the things that they did well and things that they could do better.
6. Sum up the role play with important points and answer any questions the participants may have.

Role Play:

[Scenario: A father who does not have time for child's birth certification and thinks it is not that important. His wife does understand the importance but lacks awareness regarding birth certification and cannot convince her husband because they do not talk about such matters.]

Step 1:

The trainer should explain the scenario to the group of participants.

Step 2:

The trainer then selects three participants from the group. Ask one of the participants to adopt a role of father, other participant, as his wife, whereas the third participant should play the role of a LHW/other frontline worker who has to convince the husband/wife.

Step 3:

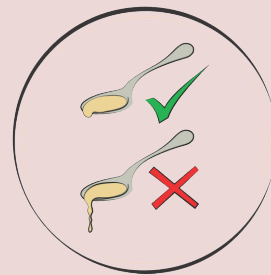
The trainer should explain the participants to use counseling card during the role play. It is important to emphasize that the participant who is playing the role of LHW/other frontline worker should have a positive attitude, show care and encouragement, listen to the parent/caregiver, and fulfil all three steps of the ABC of counseling.



KEY MESSAGES

- Getting a child registered is a legal obligation and a fundamental right of the child. It is a legal proof of child's birth, parentage and place of birth. With this registration the child will be able to have an identity and can avail other rights and services as well.
- In Pakistan, one can obtain birth registration either from birth certification authority or form National Data base and authority (NADRA). Parents/caregivers should register their babies at the nearest union council/village councils or through digital birth registration.
- Birth registration has many benefits;
 - It establishes citizenship and national identity and offers protection against exploitation including, child marriage, child labor, trafficking.
 - It facilitates access to basic services like healthcare, child protection and education.
 - It also facilitates future employment and financial assistance.

Practice 9



COMPLEMENTARY FEEDING

PRACTICE 9:

Starting at six months of age, feed children with freshly prepared and hygienic variety of complementary food, while continuing to breastfeed babies up to two years or longer.

Learning objectives:

By the end of the session,

- Participants will be able to state when to introduce complementary food to babies.
- Will be able to describe the importance of feeding variety of food.
- Will be able to explain the importance of early stimulation and responsive care giving while feeding the babies.



Methodology:

presentation, brainstorming, discussions.



Material needed:

power point slide, handouts, flip chart and markers.



Time duration: 1 hour

Activity 1:

1. Ask the participants what is their common observation regarding child nutrition after 6 months?
2. When do people normally start adding semi-solids in to child's diet, other than breastmilk.
3. Now ask the participants to brainstorm for some nutrient rich homemade recipes for the child.
4. Write down important points on a flip chart.
5. Tell them that after 6 months, a child needs energy-rich additional food along with breastfeeding.
6. Begin the session with the help of slides.

SLIDES

Slide 1: Complementary feeding

- Start introducing liquids and semi liquids after 6 months
- small portions of meal 4-5 times a day and breastfeeding
- Variety of nutrient rich food items
- Freshly home made food
- Ensure hygiene and cleanliness
- Continue breastfeeding up to 2 years
- Proper meals after 2 years of age•

Slide 2: Barriers to complementary feeding

- Lack of resources
- No family support
- Demanding child
- Junk food

Slide 3: What LHW/other frontline worker can do

- Talk to woman/family
- Guide on nutrient rich homemade recepies



KEY MESSAGES

- After 6 months, breast milk alone does not fulfil child's nutritional requirements, Therefore, additional food should be introduced to child's diet.
- Around 6 months, a child is ready to take additional food other than breastmilk. If not given on time or inappropriately, the growth of an infant can be affected can cause malnutrition and stunting.
- At 6 months, a child requires smooth transition from breastmilk to semi-solids and ultimately to solid foods. First introduction should be semi solids, like yogurt, kheer, firni, and porridge. At 9 months, finger foods like home-made potato chips, slice of any fruit or vegetable like apple, banana, carrot etc., and mashed food can be added to yogurt etc. By the age of 2 years, a child learns to self-feed and should be given everyday normal meals like roti with gravy, etc.
- It is important to emphasize that the meal given to child should be healthy and contain all important nutrients. Eggs, meat, beans, fish, and fresh vegetables and fruits are some of the good examples.
- Increase the consistency and quantity of food with time. In the beginning, the food should be neither too thick nor too thin. Similarly, an infant should be given 2-3 meals per day at the age of 6-8 months, whereas at 9-23 months, a child should be given 3-4 meals per day along with 1-2 additional snack items. Continue breastfeeding the child till 2 years.
- Also add fortified foods or vitamin-mineral supplements in child's diet whenever needed.
- During illness, increase the fluid intake along with breastfeeding.
- Parents/caregivers should ensure that clean and safe water is used to prepare food.
- Caregivers should wash their hands with soap and water before feeding babies and also wash the babies hands.
- Caregivers should talk to the babies while feeding and respond to them as they make sound or talk to you.
- Fathers should play an active role in feeding the babies.
- Avoid giving your child unhealthy market made unhygienic food items, like soft drinks, junk food items like candies, chips etc.

NOTE FOR THE FACILITATORS

Preparation of food:

1. Pureed fruit

Ingredients	Method
Fresh apple, pear, peach, apricot or dried prunes (stones removed)	If using fresh fruit: wash, peel, core and dice it. If using dried fruit: wash and dice it, then soak it in just enough water to cover it for at least 15 minutes before cooking. Place fresh fruit or soaked dried fruit and any remaining liquid in a saucepan. Add just enough water to cover the bottom of the saucepan and cook quickly until fruit is soft. Press pulp through a strainer or puree in a blender.

2. Banana rice pudding

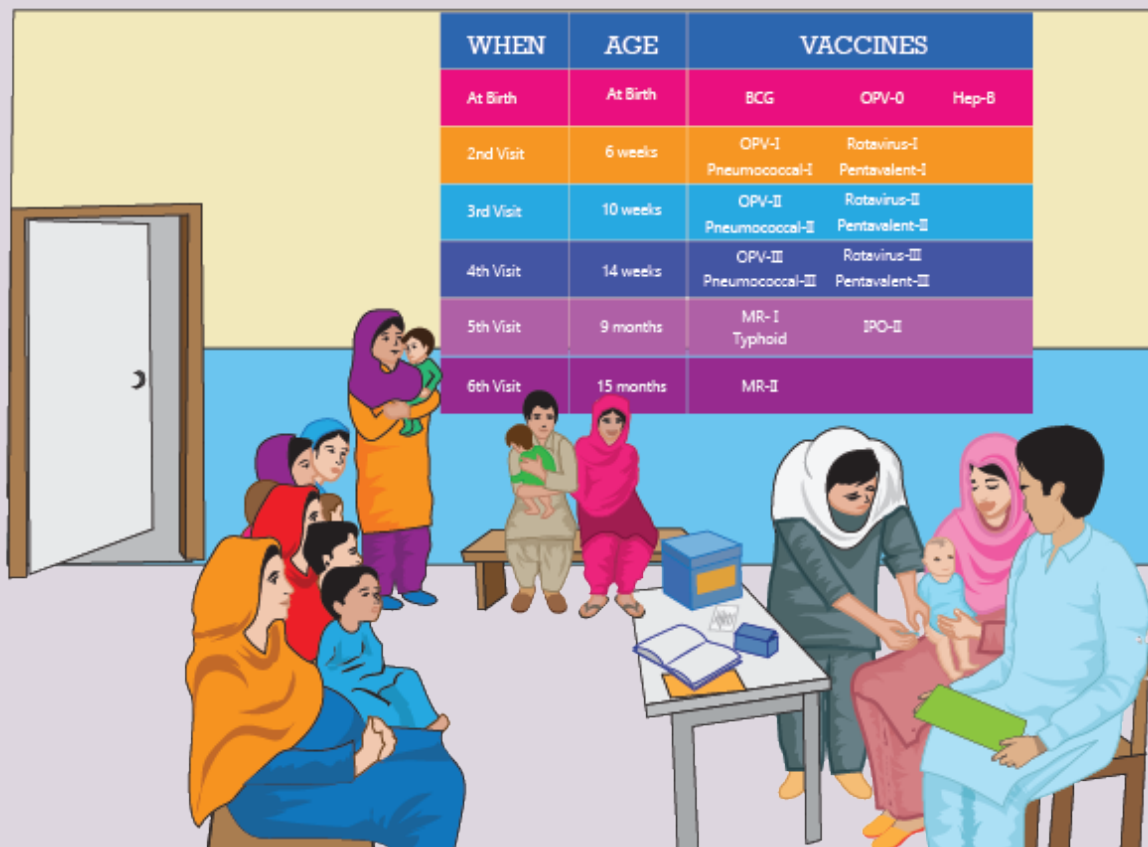
Ingredients	Method
3/4 cup cooked rice 3/4 cup (180mL) expressed breastmilk, formula or full-cream milk (if over 12 months) 1/2 banana, mashed	Mix together the cooked rice, milk and banana. Heat in a saucepan over low heat until milk is absorbed, stirring frequently. Cool and serve warm or cold.

3. Mix vegetables kichdi

Ingredients	Method
Rice – ½ cup Moong dal – ½ cup Mixed vegetables washed and chopped (potato, carrot, peas, beans) – 1 cup Ghee – 1 tsp Turmeric – a pinch Cumin seeds – ½ tsp	Clean and wash the dal and rice, and soak them in water for half an hour. Heat the ghee in a cooker with the cumin seeds until the seeds crackle, then add the other ingredients and water. Let this mixture cook for 4 whistles, and then mix it with a spoon and give it to your baby.

Practice 10

WHEN	AGE	VACCINES		
At Birth	At Birth	BCG	OPV-0	Hep-B
2nd Visit	6 weeks	OPV-I Pneumococcal-I	Rotavirus-I Pentavalent-I	
3rd Visit	10 weeks	OPV-II Pneumococcal-II	Rotavirus-II Pentavalent-II	
4th Visit	14 weeks	OPV-III Pneumococcal-III	Rotavirus-III Pentavalent-III	
5th Visit	9 months	MR-I Typhoid	IPO-II	
6th Visit	15 months	MR-II		



VACCINATION

PRACTICE 10:

Take all girls and boys for vaccination at birth, 6 weeks, 10 weeks, 14 weeks, 9 months and 15 months for full course of immunization to protect them from 12 diseases.



Learning objectives:

By the end of the session,

- Participants will be explain the importance of immunization.
- Will be able to state the number of times children need to be vaccinated.



Methodology:

presentation, brainstorming, discussions, video.



Material needed:

power point slide, handouts, flip chart and markers.



Time duration:

1 hour

Activity 1:

1. Ask the participants if they have taken their sons and daughters for vaccination.
2. Ask them how many times the children need to be vaccinated.
3. Ask if they keep immunization cards with them?
4. Ask what could be some of the reasons children are not vaccinated.
5. They may say that people think it is a western conspiracy or because hospitals or facilities are far away, or because people think frequent vaccination is not required.
6. Listen to these barriers carefully and write them on a flip chart and add your points as well.
7. Now ask them to brainstorm for the solution of these problems.
8. Encourage and allow every participant to speak.
9. Find possible solutions and write them down next to their relevant problem on the flip chart.
10. After brainstorming, select participants for role play.
11. Write down important points on a flip chart.
12. Tell them that every child should receive 6 vaccines from birth till 18 months of age and it is safe to immunize children even when they are sick.
13. Write their responses in the flip chart and summarize the key points with the help of the slides below:

SLIDES

Slide 1: Vaccinate all children as scheduled

- Prevents from 10 life threatening diseases
- Vaccination is free
- Not harmful for babies
- Consult a doctor in case any symptoms appear

Slide 2: Common barriers around vaccination

- Religious beliefs
- Western conspiracy
- Vaccine hesitancy
- Far flung areas
- No family support

Slide 3: Role of a LHW/other frontline worker

- Talk to the mother/family
- Ensure safety of a vaccination card
- Help learn the schedule of child's immunization

Activity 2:

1. Divide the participants into three groups and ask them to do a role play.
2. After the role play, appreciate the effort of the 3 participants who did the role play.
3. Ask them about their performances, specifically the role LHW/other frontline worker. Was she able to convince the family? Did she make proper use of counseling card? What did she do best and what she could do differently? Listen to her views.
4. Now ask other participants about their views.
5. Listen to their points carefully and note down important points on a flip chart. Add your points in the end and explain to the participants about the things that they did well and things that they could do better.
6. End the discussion with revision of important points.

Role Play:

[Scenario: A family whose men think vaccination is a western conspiracy]

Step 1:

The trainer should explain the scenario to the group of participants.

Step 2:

The trainer then select 3 participants form the group. Ask one of the participants to adopt a role of a father, second participant should play the role of a grandmother and the third act as a LHW/other frontline worker who has to convince them.

Step 3:

The trainer should explain the participants how to use counseling card on vaccination during the role play. It is important to emphasize that the participant who is playing the role of LHW/ other frontline worker should discuss all relevant points, have a positive attitude, show care and encouragement, listen to the parent/caregiver, and fulfil all three steps of the ABC of counseling.

KEY MESSAGES

- Immunization prevents a child from deadly diseases and strengthens child's ability to fight with these diseases. It also ensures child health and development.
- A child should be given 6 doses of vaccines in total, starting right after birth till 15 months of child's age. Some additional doses can be given at a later stage.
- These vaccines are the cheapest way of preventing childhood diseases. Every child should receive recommended vaccinations and complete their vaccination course. Parents/family should ensure that their child receives all immunizations.
- Interact with the child by talking, playing or give him a toy to distract him from crying after vaccination session.
- There are two types of immunization vaccines; injectable and oral. The schedule for child immunization is as follows:

When	Age	Vaccine
1st	At birth	BCG, OPV-0, Hep-B
2nd	At 6th week	OPV-I, Pneumococcal-I, Rota virus-I, Penta-I
3rd	At 10th week	OPV-II, Pneumococcal-II, Rota virus-II, Penta-II
4th	At 14th week	OPV-III, Pneumococcal-III, IPV, Penta-III
5th	At 9 months	Measles- I
6th	At 15 months	Measles-II

NOTE FOR THE FACILITATOR

These vaccines provide protection against 10 deadly diseases. These diseases and their causes are:

1. **Tuberculosis:** It is usually spread through breathing and can harm lungs and brain, if not treated on time.
2. **Polio:** Pakistan is one of the third countries with poliovirus cases. Polio is a highly infectious viral disease which attacks the nervous system and can cause permanent paralysis, especially in lower limbs. Polio is spread through contaminated water, food, human saliva or faeces.
3. **Diarrhea/Rota Virus:** Rota virus is the number one cause of diarrhea. If left untreated, Rota virus can cause dehydration which is a leading cause of death in children. Every year 53,300 children die in Pakistan because of this virus. Its symptoms include fever, belly pain, vomiting, watery stools.
4. **Pneumonia:** It is a lung infection that can also cause death in infants. It spreads through breathing and causes inflammation which results in stiffness of lungs and makes it difficult to breathe.
5. **Pertussis (Whooping cough):** Pertussis causes violent cough that makes breathing difficult and can also harm child's internal organs if untreated. It is spread from one child to another through cough, sneeze or by touching infected areas.
6. **Tetanus:** It is an infectious disease caused by bacteria found in soil. It enters the body through wounds, cuts or burns. If untreated, it can cause throat paralysis and affect the respiratory muscles which can lead to death.
7. **Measles:** Measles is an infectious viral disease which is highly contagious. It can spread through sneezing, coughing and physical contact. Its symptoms include fever, respiratory involvement and rash. If not treated, this disease can be fatal.
8. **Meningitis:** Meningitis is the inflammation of the membranes that protect brain and spinal cord. It can be bacterial, viral or fungal. Symptoms include fever, vomiting, severe muscle pain, stiff neck and rash. If left untreated it can result in death.
9. **Hepatitis-B:** This disease is caused by hepatitis B virus which causes a contagious liver disease. It can be transmitted from person to person through exchange of blood or other body fluids. It can also be transmitted from an infected mother to her child at birth. Its symptoms include jaundice, nausea, vomiting, abdominal pain and loss of appetite.
10. **Diphtheria:** Diphtheria is a contagious bacterial disease. It can spread from person to person and attacks the respiratory system. If not treated on time, this disease can affect the internal organs that can result in death.

Practice 11



ADOLESCENT & MATERNAL NUTRITION

PRACTICE 11:

Ensure that children, adolescents and women receive adequate amounts of micro nutrients (vitamin A, iron, zinc and folic acid) either in their diet or through supplements.



Learning objectives:

By the end of the session,

- Participants will be able to explain the importance of micronutrient for women and children.
- Will be able to understand what food can be rich in micronutrient.



Methodology:

presentation, brainstorming, discussions.



Material needed:

power point slide, handouts, flip chart and markers.



Time duration:

1 hour

Activity 1:

1. Ask the participants what type of nutrition a woman should have during pregnancy.
2. Ask about possible barriers in the way of good nutrition and use of iron folic.
3. Listen to the participants carefully.
4. If they say that women cannot take iron folate because they feel nauseated or women cannot take care of their nutrition because they cannot afford good quality nutrition. Ask them what could be the solutions.
5. Write their responses on a flip chart (they may mention milk, iron, meat, fruits etc.).
6. Ask them what kind of nutrition and micro-nutrients should children and adolescent receive.
7. Tell them that it is the role of a front line worker to facilitate and help women in identifying problems and their possible solutions.
8. Explain with the help of the slides how nutrition in pregnancy is linked with child's physical and mental development.
9. Emphasize that pregnant or lactating mother should take adequate amount of food including carbs, meat, milk, fruits & vegetables along with iron folate.

SLIDES

Slide 1: Maternal nutrition and its benefits

- Child's development starts from the womb
- Nutrient rich diet during pregnancy ensures healthy child
- Link of nutrition with child growth and development

Slide 2: Role of Iron folic

- Iron intake- healthy baby
- Helps in child's brain development
- Treats Anemia

Slide 3: Expected obstacles

- Lack of resources/ awareness
- Nausea and vomiting
- Side effects of iron
- No support from family

Slide 4: Guide the mother/family

- Food alternatives
- Use iron with water
- Talk to the family
- Suggest nutrient rich recipes



KEY MESSAGES

- Children and pregnant women must take foods that are rich in minerals, vitamins, iron, fiber, fatty acids and iodine. Micronutrients produces enzymes and hormones that play an important role in proper growth and developments. These micronutrients:
 - Help in the development of bones and metabolism.
 - Develop immunity in the body against germs and infections.
 - Help the body grow.
 - Prevent goiter.
 - Prevents anemia and helps in brain development.
 - Strengthen eyesight.
- It is also advised to deworm your children. Deworming helps the body get rid of parasites such as roundworms and tapeworms and improves mental and physical development.
- Taking nutritious diet and use of iron folate during pregnancy is important for both mother and her baby. A woman is recommended to take proper nutrition as advised by her doctor/ nutritionist.
- Iron helps in the development of baby's brain cells and provides a positive boost to brain development.
- During pregnancy, a woman should take foods that are rich in iron, proteins, iodine, zinc, and vitamin C & D. Intake of foods such as, meat, fish, milk, fruits and vegetables is recommended.
- Families who have limited resources can always use alternatives. For example, women who cannot afford meat can use legumes and beans. Similarly, leafy green vegetables are a good alternative for iron.
- Iron deficient diet of the pregnant mother can result in low birth weight baby and also increases the chances of diseases in the newborn.
- Iron helps in reducing fatigue, weakness, and anemia; which can cause complications during delivery.
- Besides the pregnant mothers, the lactating mothers should also use iron folate at least for first 6 months.
- Children and adolescent girls should also be fed micronutrient rich food.

Practice 12



USE OF BED NETS

PRACTICE 12:

Protect children and pregnant women in malaria and other viruses endemic areas by ensuring that they sleep under insecticide –treated bed nets.



Learning objectives:

By the end of the session,

- Participants will be able to explain the importance of sleeping under insecticide treated bed nets (ITN).



Methodology:

presentation, brainstorming, discussions.



Material needed:

power point slide, handouts, flip chart and markers.



Time duration: 30 mins

Activity 1:

1. Ask the participants if families in the community use bed nets and if so who sleeps under it.
2. Tell them that it is important to keep houses clean and in areas where Malaria is common, pregnant woman and children should sleep under insecticide.
3. Ask about possible barriers in people using ITNs in the house.
4. Ask them to brainstorm for the possible solutions of these problems.
5. Write down their suggestions on the flip chart.
6. With the help of slides, discuss malaria and benefits of bed nets in detail.
7. Summarize the main points and answer any questions the participants may have.

SLIDES

Slide 1: Diseases spread by mosquitos

- Life threatening
- Can be controlled
- Dengue & Malaria

Slide 2: Hanging and using a bed net

- Demonstrate how to set a bed-net
- Important to demonstrate to the participants

Slide 3: Barriers in using bed nets

- Lack of awareness
- Unavailability of bed nets
- Poverty

KEY MESSAGES

- Malaria and Dengue, both spread through mosquitoes and their symptoms include fever, vomiting and diarrhea, headaches, fatigue and muscular pains. Malaria or dengue viruses are passed on from one person to another person through a mosquito bite.
- Mosquito infested diseases are life threatening and therefore it is important to take precautionary measures. Using a bed net ensures safety from these deadly diseases.
- Protection from these deadly diseases is very important, especially for pregnant women, babies and children. Malaria and Dengue can cause anemia and can damage heart, lungs and liver. They can also cause miscarriage and premature birth in women. Therefore, it is advised that people, especially pregnant women and children should sleep under bed nets.
- It is advised to keep houses, drains and vegetation around the house always clean. Avoid containers and objects where water can be stored and allows mosquito breeding. Always keep water pots covered.
- Remember! These deadly diseases can kill. It is advised to consult a doctor as soon as a symptom appear.

Practice 13



TAKING CARE OF A SICK CHILD

PRACTICE 13:

Continue to feed and offer more fluids including breast milk when children are sick and wash hands before feeding.



Learning objectives:

By the end of the session,

- Participants will be able to describe how to take care of a sick child at home.
- Participants will be able to explain the danger signs in children and when to take action to seek health care and treatment.



Methodology:

presentation, brainstorming, discussions.



Material needed:

power point slide, handouts, flip chart and markers.



Time duration: 1 hour

Activity 1:

1. Divide the participants into two groups and ask them to brainstorm and write responses on a flip chart to the following questions:
 - What kind of diseases or sickness do children have?
 - Where do caregivers take their children if they are sick?
 - When should the child be taken to the health facility?
 - What are some home remedies to treat a child with diarrhea or pneumonmia?
 - How a sick child should be treated in the family?
2. Ask them to present what they have written on the flip chart.
3. Ask them to brainstorm for possible solutions and tell them that is it their responsibility as front line workers to guide the mother/family for possible solutions.
4. After the exercise, prepare the participants for a role-play.
5. Summarize the responses with the help of the slides below:

SLIDES

Slide 1: Proper home treatment for mother and her child if they have

- Cough
- Fever
- Respiratory infections
- Diarrhea

Slide 2: Requirements of a sick child

- Sick child is more vulnerable to diseases
- Take extra care of hygiene
- Feed the child frequently along with breastfeeding
- Engage in different stimulation activities
- Consult a doctor
- Timely identification of signs

Slide 3: Micronutrients and deworming

- De-worm at least twice a year
- supplementation for both mother and her child

Slide 4: Role of a LHW/OTHER FRONTLINE WORKER

- Talk to woman/ family
- Guide how to check temperature
- Guide how to make ORS



KEY MESSAGES

- Parents/family can deal with simple cases of illnesses at home and monitor the recovery of the patient.
- Pregnant women and children are more likely to suffer with illnesses than can be treated with simple home treatment.
- A child needs special care and treatment during illness. When the child is not eating or feeding, or not active, it is more likely that child is sick. The mother/family should find out why the child is not participating in eating or feeding activities.
- It is important to keep an eye on child's health and provide them with timely home treatment. Consult a doctor in case of emergency. Some of the common signs of illness are;
 - High fever.
 - Loss of appetite.
 - Sweating.
 - Persistent cough and difficulty in breathing.
 - Vomiting.
 - Diarrhea.
 - Unnecessary crying.

NOTES FOR THE FACILITATORS:

Home treatment for high fever: In case of high fever, remove child's clothes and cool the body with the help of a piece of cloth soaked in luke warm water. This will help lower the body temperature. Also, give more fluids to your child and breastfeed repeatedly in case of babies. If the baby/child has convulsions/fits or is unable to drink or is abnormally sleepy, take him/her to a health facility immediately.

Home treatment for Diarrhea: Diarrhea is passing of watery stools 3-4 times in one day and one of the main reasons of death in young children. It drains water and nutrients from the body which leads to dehydration. Oral Rehydration Salts (ORS) and Zinc is the best way to treat this illness. Give one cup of ORS to your child after every stool. Always keep ORS and Zinc packets at home. It is also important to keep the house clean and ensure proper sanitation. Wash hands before and before and after cooking, feeding child, and using latrine.

How to prepare ORS:

- Wash hands with soap and clean and safe water.
- Take a clean jug, cup, spoon, a packet of ORS and clean boiled cool water. Mix the ORS sachet in 4 cups of water.

Home treatment for Cough:

In case of cough, give your child more fluids like warm water with honey, herbal tea etc. It helps soothe the throat and hence reduces the intensity of coughing. Don't give any medicine until and unless prescribed by a doctor. Take your child to the hospital immediately, if the child starts breathing faster than usual or has difficulty in breathing.

Practice 14



STIMULATION AND PLAY

PRACTICE 14:

Support children's mental and social development, by responding to their needs and by stimulating them through talking, playing and providing healthy interactions and environment.



Learning objectives:

By the end of the session, participants will be able to:

- Understand the five key areas of development: Physical, intellectual, language, social and emotional domains.
- Identify activities that can improve the different domains areas or what parents or caregivers should be doing during an engagement with the child geared at enhancing a particular domain.
- Understand the value of stimulation and play when engaging with children.
- Explain how parents and care givers can interact with children.
- Have skills to play with babies and young children.



Methodology:

presentation, brainstorming, discussions.



Material needed:

power point slide, handouts, flip chart and markers.



Time duration: 1 hour

Activity 1:

1. Divide the participants into three groups and ask them to brainstorm on how parents/care giver interact with young children.
2. Ask them what their understanding is of cognitive development.
3. Ask them if there is a culture of talking and playing with children.
4. Ask them to write responses on a flip chart.
5. Ask them if there is any barriers in playing and talking with children.
6. Listen to them carefully.
7. Now ask the participants what factors or activities are important for child's better mental development.
8. If they mention love, care, play etc., appreciate them and write down important points on the flip chart.
9. With the help of the slides explain to the participants that all children should be given a favorable environment where they can play.

Activity 2:

1. Tell the participants at the end of the first day that they will have an assignment that they will do at home with their children. Share the questions to every participants to take home.
2. Ask participants to identify traditional/local games that parents/caregivers can involve their children of different ages 0-6 months 6-24 months: 2-3; 4-6 years.; Name of play activity and material required for the play activity. (Give this as homework so that participants can ask children at home or neighbours)
3. Identify traditional children songs rhymes and lullabies.
4. Divide the participants into 6 groups.
5. Each group needs to fabricate a play object for babies, toddlers, children with colours and shakes from material available in their homes.
6. Mention to participants that everyone will have a chance to fabricate their own toys from different safe materials at home.
7. Discuss, the different types of toys and safety issues of materials.
8. Summarize and conclude the session.

SLIDES

Slide 1: What is early childhood development?

- A holistic approach- physical and mental development
- First 2 years are crucial
- Begins with pregnancy
- Responsive caregiving
- Favorable environment

Slide 2: Significance of early childhood development

- Better health
- Better individual
- Better future
- Better economy

Slide 3: Five main areas of development

- Physical/Motor: (Fine motor & gross skill) development
- Cognitive development
- Speech and language development
- Social development
- Emotional development

Slide 4: Milestones and skills at different ages

- First 6 weeks
- 1 ½ -3months
- 3-6 months
- 6-9 months
- 9-12 months
- 1-2 years

Slide 5: Learning through Play • An interesting way of learning • Activates brain • Children love to play • Family members play a vital role	Slide 6: Barriers to play and Stimulation • Lack of awareness/ resources • Unhealthy environment • Tense relationships/ No family support • No family planning
Slide 7: LHW/other frontline worker is the guide • Talk to the mother/family • Guide on how to make homemade toys from available resources	Slide 8: Play and Stimulation activities for children 0-6 months • Skin to Skin contact • Talk to your child • Show love and care • Copy sounds • Introduce various textures • Rattles • Peek-a-Boo

Activity 2:

1. Make 4 groups with 4 participants each and give each group a toy and ask them how these can be utilized for play and stimulation.
2. Ask them what a child can learn from these toys/utensils?
3. Call every group one by one and ask them to share their understanding of toy utilization. Add your points in the end.
4. Explain that this activity of using a simple toy will be helpful to activate their creative thought process and help them convince mother/family in a better way.

Role Play:

[A family who thinks that children learn in schools and a child cannot comprehend anything before age 5. Also, nobody has the time to play with the child because everybody is busy with work; father is always unavailable, mother has to do household chores, grandmother looks after the baby but she thinks she is too old to play with the child.]

Step 1:

The trainer should explain the scenario to the group of participants.

Step 2:

The trainer then selects 4 participants from the group. Ask one of the participants to adopt the role of father, second participant should act as a MIL, third participant adopt the role of mother and fourth participant should perform as a LHW/other frontline worker who has to convince that family.

Step 3:

The trainer should explain the participants to use counseling card during the role play. It is important to emphasize that the participant who is playing the role of LHW/other frontline worker should have a positive attitude, show care and encouragement, listen to the parent/caregiver, and fulfil all three steps of the ABC of counseling.

- After role play, appreciate the effort of the four participants who did the role play.
- Ask other participants about their views.
- Listen to their points carefully and note down important points on a flip chart.
- Explain to the participants about the things that they did well and things that they could do better.
- Sum up the important points.



KEY MESSAGES

- Respond to your child's sounds and interests. Call the child's name, and see your child respond.
- Encourage your child to talk and answer your child's questions.
- Teach your child stories, songs and games. Talk about pictures or books. Sample toy: book with pictures.
- Help your child count, name and compare things. Make simple toys for your child. Sample toys: Objects of different colours and shapes to sort, stick or chalk board, puzzle.
- Give your child clean, safe household things to handle, bang, and drop. Sample toys: containers with lids, metal pot and spoon.
- Help your child to talk. Show and talk about nature, pictures and things.
- When children participate in ECD activities their overall development is enhanced.
- Fathers should take an active role in playing with children, taking them outdoors to play, responding to their needs etc.
- Grandparents and other members of families can also interact with children.

NOTES FOR THE FACILITATORS:

- Child development is a process every child goes through.
 - During this process, a child's mental, physical, creative social and emotional development occurs and the child learns various skills like sitting, talking, communicating, etc. These skills are called developmental milestones. These developmental milestones are divided into 5 categories;
1. Physical development:
 - a. Fine motor skill development

This includes a child's ability to grasp or hold things by using small muscles like fingers, hands or wrists. For example, holding a spoon or plate, holding a pencil and writing, turning the pages of a book, etc. Fine motor skills develop after gross motor skills.
 - b. Gross motor skill development

It deals with a child's ability to make larger movements by using arms, legs, body, etc. Crawling, jumping, walking and running are a few examples of gross motor skills.
 2. Cognitive Development:

Cognitive development is the building of thought processes, including thinking, remembering, problem solving, and decision-making. It helps a child understand about self and the surrounding environment. Brain development is a part of cognitive development.
 3. Speech and language development:

It deals with child's, ability to use and understand language. This includes using of words, talking, reading and communicating. By talking and reading to the child caregivers can help the child learn this skill faster.
 4. Social and Emotional Development:

It deals with child's ability to interact with people and surrounding. It includes a child's ability to develop relationships, experiences, expressions, and management of emotions while dealing with others. Some of the examples are smiling waving, sharing, etc.

 - The early years, especially the first 2 years of a child's life are very crucial. This is the time when a child is growing and learning quickly. By providing a child with proper nutrition along with love and care and opportunities to explore and learn will not only improve a child's physical health but will also enhance his/her mental capabilities.
 - Play & stimulation helps stimulate a child's brain and it is something that a child needs. It is an interesting way of learning and helps activate the brain and improves child's physical and mental capabilities. Parents can play an important part and can teach their children different things through stimulation and play. It also develops and strengthens the bond between child and parents.

Practice 15



MEN'S ENGAGEMENT

PRACTICE 15:

Ensure that men actively participate in providing nurturing and care to children and are involved in reproductive health initiatives.

Learning objectives:

By the end of the session,

- Participants will be able to describe how fathers can be engaged in taking care of the pregnant mother as well as of children.
- Participants will be able to state the importance of men's engagement in the upbringing of children.



Methodology:

presentation, brainstorming, discussions.



Material needed:

power point slide, handouts, flip chart and markers.



Time duration: 40 mins

Activity 1:

1. Ask the participants about the role of husband during pregnancy, child birth and child care.
2. Ask them about their experience; do husbands take interest in taking decisions for their women, are they supportive?
3. Now ask them how important is a father's involvement in his child's upbringing?
4. Now talk about the possible barriers and ask the participants about what majors barriers to male involvement can be?
5. They may say that mostly men lack reproductive knowledge or any other knowledge regarding maternal and child health, or, men don't have awareness regarding family planning and its various methods, or, if participants mention cultural beliefs or spousal communication as a barrier, appreciate them. Write down these important points on a flip chart.
6. Then lead the group to brainstorm on how husband's involvement can be ensured in offering care to women during pregnancy and child birth.
7. Ask them to problem solve for these barriers.
8. Listen to their responses carefully and write their responses on a flip chart.
9. If they say that a father's role is as important as mother's, appreciate them.
With the help of slides talk about the role of husbands in offering care to women during pregnancy and after childbirth.

SLIDES

Slide 1: Importance of Husband's involvement in offering care to women during pregnancy and child birth

- Participation of husband leads to a better understanding between husband and wife.
- Importance of husband in raising a healthy and happy family
- Family Planning- a mutual decision

Slide 2: Barriers to male involvement

- Lack of knowledge about care during pregnancy
- Misconceptions and lack of awareness about the use of family planning methods
- Poor communication between partners
- Cultural beliefs

Slide 3: Role of LHW/other frontline worker in ensuring Husband's involvement

- Know the cultural and traditional practices that hinder male involvement
- Know how to sensitize and educate mothers/families
- Know why it is important for husbands to get involved



KEY MESSAGES

Men should play an active role by;

- Taking women to hospital for regular check-ups during pregnancy.
- Ensuring that a birth preparedness plan is in place.
- Emotionally supporting women during pregnancy, childbirth and in child rearing.
- Ensuring a safe environment for their forthcoming child.
- Ensuring that women get enough food and rest during pregnancy.
- Men should help in feeding the child, playing with him/her and interacting with the child.

NOTES FOR THE FACILITATORS:

- A woman needs extra care and support during pregnancy and after childbirth. A husband should accompany her for her antenatal checkups, provide her with healthy nutrition and take care of her physical, emotional and nutritional requirements during this time.
- Only a woman with healthy psychosocial wellbeing can deliver a healthy baby. A woman cannot give her child the full attention until and unless her wellbeing is taken care of and this is only possible when she has support from her husband.
- It is also important that partners should communicate on everyday matters. Having a regular conversation about family matters is a sign of a healthy relationship. Mutual understanding and decision making ensures child's better upbringing and happy family.

Practice 16



MATERNAL MENTAL HEALTH AND WELLBEING

PRACTICE 16:

Seek appropriate care and social support during pre-natal period to manage maternal mental health.



Learning objectives:

By the end of the session,

- Participants will be able to understand how mothers mental health needs to be taken care of during pregnancy, child birth and in postnatal period.
- Participants will be able to describe the importance of appropriate care and social support to the mother.



Methodology:

presentation, brainstorming, discussions.



Material needed:

power point slide, handouts, flip chart and markers.



Time duration: 40 mins

Activity 1:

1. Ask the participants if they are aware of mental health issues that women who are pregnant and recently delivered go through.
2. Ask them write on a flip chart any signs and symptoms that may indicate mental health issues.
3. Ask them what is usually done if women go through mental health issues and where they seek care.
4. Ask them if there is stigma around mental health issues in the community?
5. Write the responses in a flip chart.
6. Have a discussion around the importance of mental health care.

Mental health or psychological well-being is the ability to function well and form and maintain relationships to be happy in your everyday life. In severe cases, some mothers suffering might lead to malfunctioning and as a result, the children's growth and development may be negatively affected. Maternal mental health problems are treatable. Mother's child care demands and household chores are often overwhelming for some women. This condition often referred also as parental burn-out might result lead to depression. This condition is often prevalent in women who have just given birth (postpartum depression) and mostly with no social support.

Barriers to seek treatment

- Stigma and negative cultural attitudes towards mental illness.
- lack of universal health coverage (UHC) for mental health.
- lack of social support system.



KEY MESSAGES

- If you are pregnant get a family member to support you.
- Your partners should support and engage in child care to reduce women's workload.
- Do yoga and/or meditation, and breathing exercises and take your child for a walk outside the home.
- Play with your children as its fun and relaxing.
- Eat properly and sleep well.
- Join a mother's group or an association of women with children.
- Talk to your SBA or Lady Health Worker in your community.

Practice 17



WATER, SANITATION AND HYGIENE (WASH)

PRACTICE 17:

Ensure every member of family uses latrine and afterwards washes hand with soap, and also after helping children using toilet and disposal of feces.



Learning objectives:

By the end of the session,

- Participants will be able to describe critical times of hand washing.
- Participants will be able to understand the importance of clean and safe water.
- Participants will be able to describe the importance of hygiene and sanitations including the teeth of young children.



Methodology:

presentation, brainstorming, discussions.



Material needed:

power point slide, handouts, flip chart and markers.



Time duration: 1 hour

Activity 1:

Divide the participants into four groups and give a task each to the groups:

1. Write the critical times of hand washing on VIPP cards.
2. Write about any barriers that communities may have to practicing hand washing, hygiene and sanitation.
3. Write about safe water handling and point of use.
4. Write about how hygiene and sanitation and toilet use.
5. Make the groups present on their ideas.
6. Summarize key points by using the slides below.
7. Answer any questions the participants may have.

SLIDES

Slide1:Critical times to wash hands

- Wash hands properly with soap and water:
before eating or feeding the baby,
after using the toilet and changing the babies nappies.

Slide 2: Importance of sanitation:

- Dispose of all faeces properly in a latrine, including that of children's
- Use toilet and keep it clean.
- Parents should train and help children in using toilets.

- Wash hands before and after feeding the baby

- Families should keep the house and surrounding areas clean

Slide 3: Safe Water

- Use clean and safe water for drinking and for washing hands
- Water can be made safe by boiling, use of chlorines or solar treatment
- The drinking water should be stored in a covered storage and taken out with clean utensil.

Slide 4: Be the Guide

- Demonstrate proper technique of handwashing

Activity 2:

1. After discussing the importance of hand washing, hygiene and sanitation, demonstrate proper handwashing technique to the participants.
2. Make the participants wash their hands by demonstrating proper hand washing technique.
3. Now talk about the barriers and ask the participants what have they observed in their community?
4. If they mention, lack of awareness, improper way of handwashing or no specific area for latrine, ask them to brainstorm on possible solution.
5. Revise important points and answer any questions the participants may have.

Activity 3: Role Play

1. Divide the participants into four groups and ask them to perform the role play.
2. After role play, appreciate the effort of the two participants who did the role play.
3. Ask other participants about their views.
4. Listen to their points carefully and note down important points on a flip chart.
5. Explain to the participants about the things that they did well and things that they could do better.
6. Sum up the role play with important points.

Role Play:

[Scenario: A mother does not take care of her hygiene or hygiene of her child. She has a lot of work to do and she cannot wash hands every time.]

Step 1:

The trainer should explain the scenario to the group of participants.

Step 2:

Select two participants from the group. Ask one of the participants to adopt a role of woman whereas the other participant should play the role of a LHW/other frontline worker who has to convince the woman on handwashing.

Step 3:

Explain the participants to use counseling card during the role play. It is important to emphasize that the participant who is playing the role of LHW/other frontline worker should have a positive attitude, show care and encouragement, listen to the parent/caregiver, fulfil all three steps of the ABC of counseling and cover all the relevant points.

KEY MESSAGES

- Wash hands with soap and water before:
 - Preparing and serving food.
 - Feeding children.
- All family members wash hands with soap and water after:
 - Using toilet.
 - Disposing off feces.
- Dispose of all faeces properly in a latrine.
- Baby faeces is more dangerous than adult feces and clean up a child who defecates immediately and dispose feces in the toilet.
- Drink clean and safe water.
- Use toilet and keep it clean.
- Parents should train and help children wash their hands and to use toilets.
- Children need to brush their teeth at least twice a day. If no tooth brush, they should use chewing sticks.
- Teach good hygiene practices like hand washing with soap, use of latrines and drinking clean water to your child at early age, he/she will pick up the good habits which will guide him/her the whole life.

NOTES FOR THE FACILITATORS:

- Washing hands is the first step towards healthy practices. Washing hands properly with soap prevents spreading of germs.
- It is advised to wash hands properly before and after eating, feeding, using latrine and disposing of faeces.
- Keeping your house clean and ensuring proper sanitation can prevent diarrhea, which is one of the most common diseases in young children.
- Babies and young children often put their hands in their mouth therefore it is important to wash their hands regularly.
- Always drink clean water and keep water pots covered.

Practice 18



GROWTH MONITORING

PRACTICE 18:

If you notice any delay or deficiency in the physical or psychological growth and development of the child, please consult nearest doctor or health worker for diagnosis and treatment.



Learning objectives:

By the end of the session,

- Participants will be able to describe the milestones of children's development.
- Participants will be able to understand what actions to take in case of slow growth in children.



Methodology:

presentation, brainstorming, discussions.



Material needed:

power point slide, handouts, flip chart and markers.



Time duration: 1 hour

Activity 1:

1. Ask participants what they understand by growth monitoring and its importance.
2. Write the responses on the flipchart.
3. With the help of the slides, explain the growth monitoring and milestones.

SLIDES

Slide 1: Be cognizant

- Monitor child's height & weight regularly
- Consult a doctor
- Provide factorable environment
- Respond to your child's needs

Slide 2: Common Barriers to Growth Monitoring

- Lack of awareness
- Doctor is responsible
- House hold chores

Slide 3: Developmental Delays and disabilities

- Know the milestones
- Love and respect the child
- Consult a doctor
- Provide child with learning and stimulation activities

Slide 4: LHW/other frontline worker WORKER is there to help

- Talk to the mother/ family
- Guide for height and weight measurement

KEY MESSAGES

- Parents/Caregivers should closely monitor the growth of the child.
- When the child is weighed, the weight should be plotted on a graph .
- Parents should consult a health worker and/or go to health facility when they notice the child is not making progress.
- Play with children. It ensures optimal development and also screens the development delay. Allow the child the freedom to sing and dance; in this way the sense of rhythm and movement is learnt by the child.
- Clap your hands and smile to show the child you recognize and support his/her efforts. This is good stimulation and it is a way of giving appropriate attention to a child with a disability. The parent/caregiver should continue with stimulation even when there is no direct response from the child.
- Don't lock the child in the house. Take the child out for a walk and play so far as she/he is not hurt. Provide the child with learning opportunities with support and time investment.

NOTES TO FACILITATORS:

- Children have similar milestones but their pace to achieve these milestones vary from child to child. Some children may be faster or slower than others.
- It is the responsibility of their parents to keep an eye on their child as they know their child best and take them to a doctor or specialist if they observe any delay or disability.
- Children with disabilities and developmental delays should be respected and supported. These children should be given equal chance to participate in activities and should be allowed to learn & play.
- Height and weight of the child should be monitored every month. Consult the specialist if no change appears in height or weight.

Check this below

Stages of Child Development and how parents can support

Age	What children can do at this age	What children need including appropriate, sensitive and responsive caregiving
Conception /pregnancy	• Fetus • Movement in womb	• Ensure recommended Antenatal visits • Seek help of health provider in case of problems and follow advice • Eat balance diet • Sing and talk to our child in womb/ listen to soft music

Birth – 3 months	• A newborn communicates through crying and can see 8-12 inches away. • Begin to smile, track people and objects with their eyes • Prefer faces and bright colors • Turn towards sound • Make eye contact, mimics sounds and open and closes hands • Discover feet and hands and at times take to mouth	• Protection from physical danger • Exclusive breastfeeding on demand/ If feeding then provide adequate nutrition and talk and make feeding a fun session for the child • Adequate health care, such as immunization, oral rehydration therapy and hygiene • Appropriate language stimulation (play and communicate with the child and tell stories through pictures for the child to see) Frequent skin-to-skin contact with child (hugging carrying etc. Take child outside the house and show nature and communicate • Motor and sensory stimulation
4 to 6 months	• Smile and laugh • Develop preferences generally to primary caregivers -parents and older siblings • Repeat actions with interesting results • Listen to the voices and start seeing colors better. • Respond when spoken to • Laugh and babble • Imitate sounds • Explore hands and feet • Put objects in mouth • Sit when propped • Roll over • Grasp objects without using thumb • Start reaching for toys and can see things in the distance. Teething also begins at this time. • Start to giggle and laugh, can roll from stomach to back and learn to sit up.	• Protection from physical danger • Exclusive breastfeeding and if feeding then provide adequate nutrition and talk and make feeding a fun session for the child • Adequate health care. • Appropriate language stimulation with reading and story telling • Motor and sensory stimulation • Provide the child with coloured toys and non-harmful objects to play • Frequent skin-to-skin contact with child (hugging carrying etc). • Take child outside the house and show nature and communicate

7 to 12 months	- Remember simple events - Identify themselves and body parts, and familiar voices - Understand their own name and other common words - Understand simple words, say first meaningful words - Explore objects and find hidden objects - Put objects in containers - Sit alone - Pull themselves up to stand and walk - Hearing is fully developed	- Protection from physical danger - Adequate proper nutrition - Adequate health care. - Appropriate language stimulation - Motor and sensory stimulation - Allow child join in little task while you supervise - Take child for a walk to nature or outside the house - Sing and dance with child or provide opportunities to listen to music and do moves - Hug your child and praise for little actions that the child does - Talk and explain things to the child
1 to 2 years	- Imitate adult actions - Speak and understand words and ideas - Experiment with objects - Walk steadily, climb stairs and run - Recognize ownership of objects - Develop friendships - Solve problems - Show pride in accomplishments - Begin pretend play	- In addition to the requirements for healthy growth of the previous years, children at this age require support in acquiring: - Motor, language and thinking skills - Developing independence - Learning self-control - Opportunities for play with other children - Health care must include de-worming - Provide problem solving toys - Send child on little errands in the house
2 to 3.5 years	- Enjoy learning new skills - Learn language rapidly - Gain increased control of hands and fingers - Act more independently	- In addition to the requirements for healthy growth of the previous years, children at this age require the opportunity to: - Make choices - Engage in dramatic play - Have increasingly complex books read to them - Sing favourite songs - Solve simple puzzles - Praise according and provide hug or carry at times just to comfort.

3.5 to 5 years	• Develop a longer attention span • Talk a lot, ask many questions • Test physical skills and courage with caution • Reveal feeling in dramatic play • Like to play with friends, do not like to lose, share and take turns sometimes	• In addition to the requirements for healthy growth of the previous years, children at this age require the opportunity to: • Develop fine motor skills • Continue expanding language skills through talking, reading, and singing • Learn cooperation by helping and sharing • Experiment with pre-writing and pre-reading skills • Provide toys to stimulate thinking • Provide opportunities to draw and use fingers and hands often
5 to 8 years	• Gain curiosity about people and how the world works • Show more interest in numbers, letters, reading and writing • Gain more confidence and use words to express feelings and cope • Play cooperatively • Develop interest in final products	• In addition to the requirements for healthy growth of the previous years, children at this age require the opportunity to: • Develop numeracy and reading skills • Engage in problem solving • Practice teamwork • Develop sense of personal competency • Practice questioning and observing • Acquire basic life skills • Attend basic education and support from parents especially the first day at school, school occasions

Practice 19



CHILD ABUSE AND NEGLECT

PRACTICE 19:

Prevent physical, psychological and social neglect, abuse and exploitation of children and women and protect them from harmful practices like child marriages.



Learning objectives:

By the end of the session,

- Participants will be able to understand what is meant by child abuse and abuse of women.
- Participants will be able to learn how to prevent abuse and how to protect children and women from being abused.



Methodology:

presentation, brainstorming, discussions.



Material needed:

power point slide, handouts, flip chart and markers.



Time duration: 1 hour

Activity 1:

1. Ask the participants what is their understanding about neglect and abuse of children and women.
2. Ask them if they have seen abuse and neglect of children and women in the community.
3. Ask them if these get reported and if not what are the barriers to report these.
4. Write down important points one by one on a flip chart.
5. Explain that women and children need to be protected from physical, sexual and psychological abuse. Use slides for detailed discussion.

SLIDES

Slide 1: Understanding neglect and abuse

- What is abuse?
- What is neglect and violence?
- Identifying neglect and abuse

Slide 2: Types of abuse & violence

- Early child marriages
- Unwanted pregnancy
- Violation of basic human rights
- Physical violence and abuse
- Verbal violence and abuse
- Sexual violence and abuse

Slide 3: Signs of neglect/abuse • Signs in children • Signs in women • Effects of neglect and abuse	Slide 4: Responsibility of family/community • Communication among family members • Mother-Father-Child Bonding • Providing safe environment • Providing emotional support • Ensuring maternal psychosocial well being • Talk to the child/woman
Slide 5: Barriers • Lack of awareness • Abusive relationships • Lack of communication between partners • Taboos attached • Negative social norms	Slide 6: Role of LHW/other frontline worker • Rights of women and children • Guide couples/families • Encourage communication • Sensitize mother/family on neglect and abuse

KEY MESSAGES

- Protect children from physical, sexual and psychological abuse and neglect and give them love and care so that they can develop and grow which helps children feel a sense of belongingness and self-confidence.
- Perpetrators of sexual violence are usually close relatives and neighbours.
- Young people and parents need to be aware and attain skills to prevent violence.
- All girls and boys should be enrolled to school with no child left behind. Children should not be allowed to work.
- Child marriage and child labour should be avoided.
- Comfort an abused child, and take the child to the nearest clinic or One Stop Centre.
- Positive discipline is mutually respectful and encouraging which is kind and firm at the same time.
- Positive discipline is effective in the long-term as it considers what the child is thinking, feeling, learning, and deciding about himself and his world.
- Positive discipline teaches important social and life skills such as respect, concern for others, problem solving, and cooperation as well as the skills to contribute to the home, school and the larger community.
- If the child does something wrong, as a parent, talk tone-to-one with the child calmly so that he understands the consequences of his action.

NOTES TO FACILITATORS:

- Intentionally mistreating a person physically, socially or psychologically in order to harm or injure that person is called abuse. There are many types of abuse;
 - Physical abuse.
 - Psychological abuse.
 - Verbal abuse.
 - Intimate partner abuse.
 - Domestic abuse.
- Violence means when a person is physically mistreated with an aim of hurting or killing. This include;
 - Rape & sexual assault.
 - Childhood sexual abuse.
 - Marital rape.
 - Human Trafficking.
 - Child marriage.
- LHW/other frontline workers/Parents should be aware of the signs that indicate abuse in women and children. These include:
 - Bruises.
 - Burns.
 - Eye injuries.
 - Difficulty in walking or sitting.
 - Withdrawn or usually quiet.
 - Signs of fear.
 - Emotional signs of abuse like anger, sadness, or low self esteem.
 - Learning difficulties.
 - Disturbing thoughts.
- Abuse and violence are very common and it is the role of the parents/ family to provide children and women a safe environment. Women should be aware of their rights and the rights of their children and should not be hesitant in seeking help.
- Parents should provide their children with an open and friendly environment and communicate on a daily basis. Children learn from their surroundings, therefore, parents should avoid aggressive behavior in front of their children.

Practice 20



CHILD INJURIES

PRACTICE 20:

Protect toddlers, young children and teach older children on how to stay safe from hazards.



Learning objectives:

By the end of the session,

- Participants will be able to understand common injuries in children.
- Participants will be able to learn how to prevent and protect children from injuries.



Methodology:

presentation, brainstorming, discussions.



Material needed:

power point slide, handouts, flip chart and markers.



Time duration: 40 mins

Activity 1:

1. Ask the participants what are the common ways that children get injured.
2. Ask them what preventive measures are taken to protect children from injuries.
3. Write the responses on the flip chart.
4. With the help of the slides explain what are the key things to consider to prevent and manage injuries at home.

SLIDES

Slide 1: Common childhood injuries and accidents

- Burns/ cuts
- Suffocation/ Choking
- Electric shocks
- High-level falls
- Animal bites
- No supervision
- Other reasons

Slide 2: What causes these injuries and accidents

- Carelessness
- No supervision
- Unsafe environment
- Domestic animals

Slide 3: Preventive measures

- Pay attention to your child
- Ensure safe and healthy environment
- Keep a first aid box at home
- Consult a doctor

Slide 4: LHW/Other FrontLine Workers should;

- Talk to mother/ family
- Help them prepare a first aid box

KEY MESSAGES

- Keep dangerous objects should be kept out of reach of children and keep sharp objects such as knives, sharp sticks, scissors and pens in a safe place.
- Keep young children away from dangerous areas such as fires and poisonous substances.
- Pay attention to your child and protect children from injury and accident, and provide treatment when necessary.
- Keep children away from rivers, sea or from getting close to fire or electric sockets in the house.
- Keep a first aid box at home and lock up medicines and other household cleaners away from the child.
- Consult a doctor immediately if the child is injured.

NOTES TO FACILITATORS:

- It is equally important that parents should provide their children with safe environment as children are often at a risk of sustaining injuries. In case of an accident, parents should be well prepared with necessary knowledge and skills. Following are a few preventive measures;
 - Keep all medicines, drugs and potentially harmful chemicals out of reach of children.
 - Keep your children away from the kitchen. Keep sharp objects like forks, knives etc., in a safe place. Also, keep the stoves or matchboxes at a level where your child cannot reach.
 - Keep anything that can suffocate a child out of child, reach like blocks, polythene bags, etc. It is also important to know about the first aid procedures and make a first aid box that should be available at all times. Some of the first aid measures are:

Burns/ Blisters

- Get the child/person away from the heat source to stop the burning.
- Cool the burn with cool running water for 20 minutes. Avoid using ice, iced water, or any greasy substances such as cream.
- Do not pop the blisters.
- Reduce sun exposure.
- Take the child/person to a hospital.

Animal Bites:

Animals are a major source of transmitting rabies which can be fatal. Parents/Families are advised to take their domestic animals for proper vaccination and keep your children away from the animals. In case of an animal bites;

- Wash the infected area gently with soap and water.
- Cover the area with a sage and sterilized cloth/bandage.
- Take the child/person immediately to the hospital.

Insect Stings:

To avoid sting bites it is advised to cover the body properly, avoid walking in grass barefoot, Keep the house clean and insect free and apply insect repellent to exposed skin. In case of an insect bite;

- Move to a safe place in order to avoid more stings.
- Wash the infected area with cool water and soap.
- Apply a cold piece of cloth to prevent swelling.
- Take the person/child to the hospital in case of emergency.

Suffocation & choking: Parents should be careful when a child learns to crawl or walk as it is the time when a child requires more attention. Many children at this age like to put everything in their mouth. It is advised that parents should keep their environment safe for their children. Remove any harmful items that can cause choking or suffocation and avoid giving food to children that are easy to choke on. In case of choking or suffocation;

- Hold the baby face-down along your thigh with their head lower than their bottom. Give them five blows firmly on their back between the shoulder blades.
- Open windows and doors to allow abundant supply of fresh air to the child.
- Give cardiopulmonary resuscitation. (CPR)
- Take the child to the hospital immediately.

Poisoning:

Always keep drugs, medicines or chemicals out of reach of children. Children mostly get poisoned by things that are not considered poisonous like shampoo, cosmetics, washing powder and detergents etc. In case of poisoning;

- Induce vomiting by putting a clean finger in the child's mouth. Do not give any medicine for vomiting.
- Clean the remaining substance around mouth.
- Rinse mouth with water.
- Take the child immediately to a nearby hospital.

Cuts and wounds:

- Apply direct pressure on the cut or wound with a clean cloth, tissue, or piece of gauze until bleeding stops.
- Gently clean with soap and warm water.
- Apply antibiotic cream to reduce risk of infection and cover with a clean cloth
- Take the child/person to a nearby health facility and follow the health workers' advice.

Practice 21



POSITIVE DISCIPLINE

PRACTICE 21:

Make sure that children are brought up with warmth, care, kind words and gentle guidance and positive discipline from parents and family members.

Learning objectives:

By the end of the session,

- Participants will be able to understand what is positive discipline.
- Participants will be able to describe how to provide guidance, love and support to children and the benefits.

Methodology:

Presentation, role plays, discussions.

Material needed:

power point slide, handouts, flip chart and markers.

Time duration: 40 mins

Activity 1:

1. Ask the participants the following questions:
 - What do you do when the child does something wrong or annoys you?
 - How are children are disciplined in the family?
 - Why do you use that form of discipline?
2. Write their responses in a flip chart.
3. Acknowledge the use of positive discipline methods and parents who speak to their children with kind words.
4. With the help of the slides explain the importance of positive discipline and the need to speak to children with kindness and love.

SLIDES

Slide 1: Corporal punishment and verbal abuse

- Children should not be hit to discipline
- Corporal punishment has a harmful effect on children
- Do not use verbal abuse with children

Slide 2: Positive discipline

- Children can be disciplined without being physical
- Positive discipline helps them to understand the situation better and helps them to realise what is wrong and what is right

Slide 3: Addressing discipline

- Be patient with your child
- Talk to them about their problems
- Help them come to solutions

Slide 4: LHW/other frontline workers should;

- Talk to mother/ family about the importance of showing love and care to children
- Help them learn how to discipline children positively



KEY MESSAGES

- Positive discipline helps a child learn self-control and with that comes self-esteem and confidence.
- Rather than use corporal punishment to discipline the children, let us use positive discipline.
- Positive discipline is a non-violent, continuous and consistent process of guiding a child to behave in a manner that is acceptable.
- Parents should avoid telling children that they are dumb, bad etc and should not compare children with each other.
- Verbal abuse causes children to feel afraid and less confident and affects the emotions of children and also leads to health problems of children.
- Corporal punishment and any humiliating or frightening punishment are forbidden, e.g., spanking, hitting, slapping, pinching, shaking, biting, or any form of physical punishment.
- Physical punishment is not only less effective but have more harmful consequences associated with unhappiness and depression later in life.
- Parents can hurt children by humiliating them or continually criticizing their behavior, pay more attention to the good behavior of the child and praise.
- Parents and care givers should be patient with children.

Practice 22



SCHOOL ENROLLMENT

PRACTICE 22:

Support children's learning, enroll and keep girls and boys in school until the age of 18.



Learning objectives:

By the end of the session,

- Participants will be able to understand the importance of sending children to early learning centres.
- Will be able to describe why children need to be enrolled and retained in schools.
- How parents can support their children to succeed in school.



Methodology:

presentation, brainstorming, discussions.



Material needed:

power point slide, handouts, flip chart and markers.



Time duration: 40 mins

Activity 1:

1. Ask the participants if children in the community are sent to early learning centres.
2. Ask them about child enrollment; what do they normally observe in their communities? Is there any gender discrimination or preference when it comes to school enrollment?
3. Ask the participants why some people do not understand the importance of education and why do they not enroll their children in schools?
4. If they mention gender discrimination, poverty, or lack of awareness, listen to them carefully and write down these issues on the flip chart.
5. Now ask them how can we resolve these issues? Help them brainstorm and generate possible solutions for these problems.
6. Listen to their suggestions and write down each solution next to its respective problem.
7. Listen to their views carefully and write down important points on a flip chart.
8. Share with them that both girls and boys have an equal right to education; it is important that they are enrolled and stay in school until the age of 18 years. Begin the session with the help of sides.
9. Answer any questions they may have.

Activity 2:

1. Divide the participants into four groups and make them do the role plays.
2. After the role play appreciate the efforts of the four participants who did the role play and ask the LHW/other frontline worker specifically about his/her performance.
3. Ask her about the things that she thinks she did right and things that could be done differently. The trainer should ask other participants about their views.
4. Listen to their points carefully and note down important points on a flip chart.
5. Give your feedback in the end and explain to the participants about the things that they did well and things that they could do better.
6. Sum up the role play with important key points.

Role Play:

[A conservative family who thinks that education is not necessary, especially for girls, and they should spend time in the kitchen, not in schools.]

Step 1:

The trainer should explain the scenario to the group of participants.

Step 2:

The trainer then selects 4 participants from the group. Ask one of the participants to adopt the role of father, second participant should act as a mother-in-law, third participant adopt the role of mother and fourth participant should perform as a LHW/other frontline worker who has to convince that family.

Step 3:

The trainer should explain to the participants to use counseling card during the role play. It is important to emphasize that the participant who is playing the role of LHW/other frontline worker should have a positive attitude, show care and encouragement, listen to the parent/caregiver, and fulfil all three steps of the ABC of counseling and make proper use of counseling card of this session.

SLIDES

Slide 1: Why education is necessary?

- Provides better prospects for future
- Shapes the future
- Gives a sense of reasoning and purpose
- Develops a sense of responsibility
- Opens new horizons

Slide 2: School enrollment

- Child learns to read and write
- It is the responsibility of a parent to make sure children are enrolled in school and retain them

Slide 3: Barriers to school enrollment

- Poverty
- Lack of awareness
- Gender discrimination

Slide 3: ECD Centres

- Early learning Centres and Kacchi classes may be available
- Children learn to socialize in these centres

Education is not necessary, skill is	- Child's overall development is enhanced by attending ECD centres - First time in primary school
Slide 4: Role of LHW/other frontline worker Sensitize mother/ family	

KEY MESSAGES

- Enroll children in early care and development or learning centres or if there are no early learning centres, send them to Kachi classes.
- When children participate in ECD activities their overall development is enhanced.
- Participate in meetings, activities etc. of the ECD centre of your child or visit the centre from time to time as that can boost the self-esteem of the child.
- Children have a right to education; it is important that they are enrolled and stay in school until the age of 18 years.
- Encourage the children to stay in school as it will not only benefit them but also the family and community.
- Supervise the children in their learning and take active interest in their school work.
- Ask your children about their day in school and work or to recount an event they attended.
- Ensure that your child eats and sleeps well.

NOTES TO FACILITATORS:

- Education plays a vital role in our lives as it provides knowledge, skill, values. It teaches us how to read and write, how to think and communicate, and how we can make our lives better.
- It is a gradual process which brings positive change in an individual's thinking and reasoning and his/her ability to achieve a better living.
- It strengthens our beliefs and helps us distinguish between right and wrong.
- Every child should be provided with education and should be enrolled in schools for formal education when a child reaches 3 years of age.
- It is a basic right of every individual; both boys and girls to receive education.
- To equip a child with education is the responsibility of his/her parents and family. They must ensure that their child is enrolled in a school.
- Without education a child cannot achieve his/her full potential. Education opens new horizons for an individual and;
 - Allows the child to have a better standard of living
 - Allows the child to explore better options and better jobs
 - Helps the child in better decision making

